# The ORM Video Training Handbook

Narration scripts and demo directions for producing AI-generated training videos
Operative Resource Manager · UNTH Ituku-Ozalla

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**What this is.** Thirty-eight ready-to-shoot video scripts, organised by staff role. Every script is written to be pasted directly into an AI video generator. Each one names the exact screen, the exact button and the exact words to speak.

**Coverage.** 38 staff roles · 165 screens · the complete perioperative journey, from booking a case to the patient leaving recovery.

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# Part 0 — Production bible

Read this once. It makes all thirty-eight videos feel like one series instead of thirty-eight strangers.

## How to use this handbook

Every video has the same four blocks. **The brief** says who it is for and what they will be able to do afterwards. **The scene table** is the shot list: what is on screen, what moves, and the exact words spoken over it. **The narration** is written to be spoken, not read — short sentences, no sub-clauses, no jargon a new porter would not recognise. **The copy-paste prompt** is a single condensed block to drop straight into an AI video generator.

Produce them in order within a part. Later videos assume the viewer has seen the earlier ones in that part, and every part assumes Part 1.

## Cast, setting and visual language

| ELEMENT | SPECIFICATION |
|---|---|
| **Presenter** | One recurring Nigerian presenter in theatre scrubs, mid-30s, warm and unhurried. The same person in every video, so the series has a face. To camera for the opening and closing only; narrating over the screen in between. |
| **Voice** | Nigerian English, measured, about 130 words per minute. Confident, never patronising. This audience are clinicians. They are not being taught medicine, only where the buttons are. |
| **Setting** | A Nigerian teaching-hospital theatre suite. Green and blue scrubs, tiled corridors, trolleys, a nurses' station with a laptop, staff using phones. Bright, clean, busy but not chaotic. |
| **Device on screen** | A **phone** whenever the task is done standing up: rosters, milestones, check-in, calling for a patient. A **laptop** for desk work: booking, reports, publishing rosters. The app is mobile-first and the videos must look mobile-first. |
| **Screen capture** | Real screens, not mock-ups. Zoom to 150% on the field being discussed. Highlight the tap target a beat *before* the finger arrives, so the eye is already there. |
| **Patient names** | Always fictional; use the ones in these scripts. Never film a real folder number. |
| **Titles** | Lower third, sans-serif, deep blue. Module name left, role right. |
| **Music** | Soft instrumental under the opening and closing only. Silent under instruction — the words are the content. |

## Rules that keep attention

> **THE FIRST EIGHT SECONDS**
>
> Never open with "In this video we will look at...". Open with **the problem the viewer has already had this week**: "The patient was ready. The theatre was ready. Nobody had told the porter." Then the promise: "By the end of this video that will never happen on your list again."

- **One idea per sixty seconds.** If two ideas are competing, cut it into two videos.

- **Show the consequence, not the feature.** Not "tap Save", but "tap Save, and the pack provider's phone buzzes in the store tonight, while there is still time to find the mesh."
- **Name real people doing real jobs.** Sister Ngozi. Dr Okeke. Emeka the porter. Abstractions do not hold attention; people do.
- **Say the cost of the shortcut.** Every module has one tempting shortcut. Name it, and name what it costs, once, plainly.
- **Close with one action** for the next working day — not a summary.
- **Stay under four minutes.** Longer is watched once and never finished.

## Part 1 — Everyone, whatever your job

Four videos every member of staff watches, from the theatre chairman to the newest cleaner.

## V1 · Why this app exists

All staff | 2 min 30 s | Watch first

### THE BRIEF

Not a features tour. This is the video that decides whether anybody watches the other thirty-seven. Its only job is to make a busy clinician believe the app is on their side.

### THEY WILL BE ABLE TO

- Say in one sentence what the ORM is for.
- Name the three things it removes: the phone calls, the waiting, the surprises.
- Know that feedback is read and acted on.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | Corridor, early morning. A surgeon holds a phone to one ear and a folder in the other hand, visibly repeating himself. Cut to an empty theatre, lights on, nobody in it. Wall clock: 09:40. | *Nine-forty in the morning. The theatre is empty, the list started at nine, and this is the fourth phone call. Is the patient ready? Has the consent been signed? Did anybody tell the blood bank? Every one of those calls is a question the hospital already knows the answer to. It just had no way of telling you.* |
| 2 · 0:20–0:45 Promise | Presenter to camera, quiet corner of the suite. | *That is the whole reason this application exists. Not to give you more forms. To stop you chasing answers the building already has. It is called the Operative Resource Manager, the ORM, and it is not really a booking app. It is the theatre's shared memory.* |
| 3 · 0:45–1:25 The journey | Animated timeline sliding left to right: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Each node lights with a small staff avatar above it. | *Follow one patient. A surgeon books her. That single act tells the pharmacy what drugs to prepare, tells the store what to pack, tells the anaesthetist she is on tomorrow's list, and tells the ward she needs a visit tonight. Nobody made a phone call. In the morning the theatre asks for her by name over the radio. The porter's phone tells him the ward. Recovery already knows she is coming. Every one of those people is looking at the same truth at the same moment.* |
| 4 · 1:25–1:55 What it removes | Three icons appear and are struck through in turn: a ringing phone, an hourglass, an exclamation mark. | *So it takes three things out of your day. The phone calls, because the information arrives before you ask. The waiting, because everyone prepares at the same time instead of one after another. And the surprises: the missing consent found at eight in the morning, the pack nobody made up, the anaesthetist nobody rostered. The app cannot fix those. What it does is show them to you the night before, while there is still time.* |
| 5 · 1:55–2:15 Honesty | Presenter, closer. | *It will not be perfect. Some of it will annoy you. When it does, tell us. There is a Feedback button on every screen and it reaches the people who build this, not a drawer. Several things you will use this week exist because somebody on your list complained about them.* |
| 6 · 2:15–2:30 Close | Theatre doors opening, list running, staff moving with purpose. Title card: | *Watch the video for your own role next. It is under four minutes, and it is the four minutes that stop the nine-forty* |

| | | |
|---|---|---|
| | ORM · UNTH Ituku-Ozalla. | *phone call. One thing to do tomorrow: open the app and find your name.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — Corridor, early morning. A surgeon holds a phone to one ear and a folder in the other hand, visibly repeating himself. Cut to an empty theatre, lights on, nobody in it. Wall clock: 09:40.
2. **Shot 2** (push in, worth animating) — Presenter to camera, quiet corner of the suite.
3. **Shot 3** (pan right) — Animated timeline sliding left to right: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Each node lights with a small staff avatar above it.
4. **Shot 4** (hold) — Three icons appear and are struck through in turn: a ringing phone, an hourglass, an exclamation mark.
5. **Shot 5** (push in, worth animating) — Presenter, closer.
6. **Shot 6** (hold, worth animating) — Theatre doors opening, list running, staff moving with purpose. Title card: ORM · UNTH Ituku-Ozalla.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Nine-forty in the morning. The theatre is empty, the list started at nine, and this is the
fourth phone call. Is the patient ready? Has the consent been signed? Did anybody tell the
blood bank? Every one of those calls is a question the hospital already knows the answer to.
It just had no way of telling you.

SHOT 2 — push in
That is the whole reason this application exists. Not to give you more forms. To stop you
chasing answers the building already has. It is called the Operative Resource Manager, the
ORM, and it is not really a booking app. It is the theatre's shared memory.

SHOT 3 — pan right
Follow one patient. A surgeon books her. That single act tells the pharmacy what drugs to
prepare, tells the store what to pack, tells the anaesthetist she is on tomorrow's list, and
tells the ward she needs a visit tonight. Nobody made a phone call. In the morning the
theatre asks for her by name over the radio. The porter's phone tells him the ward. Recovery
already knows she is coming. Every one of those people is looking at the same truth at the
same moment.

SHOT 4 — hold
So it takes three things out of your day. The phone calls, because the information arrives
before you ask. The waiting, because everyone prepares at the same time instead of one after
another. And the surprises: the missing consent found at eight in the morning, the pack
nobody made up, the anaesthetist nobody rostered. The app cannot fix those. What it does is
show them to you the night before, while there is still time.

SHOT 5 — push in
It will not be perfect. Some of it will annoy you. When it does, tell us. There is a Feedback
button on every screen and it reaches the people who build this, not a drawer. Several things
you will use this week exist because somebody on your list complained about them.

SHOT 6 — hold
Watch the video for your own role next. It is under four minutes, and it is the four minutes
that stop the nine-forty phone call. One thing to do tomorrow: open the app and find your
name.
```

VIDEO 1 - WHY THIS APP EXISTS (2 min 30 s)
STYLE: Nigerian teaching hospital theatre suite, warm documentary tone, staff in green and blue scrubs. Recurring Nigerian presenter, mid-30s, in scrubs.
SCENES:
1 (0:00-0:20) Corridor at dawn. Surgeon on the phone, folder in hand, repeating himself. Cut to an empty lit theatre, clock at 09:40.
2 (0:20-0:45) Presenter to camera, quiet corner.
3 (0:45-1:25) Animated horizontal timeline: Booking, Pre-op visit, Packs and drugs, Send for patient, Reception, Holding, Theatre, Recovery, Ward. Nodes light in sequence with staff avatars.
4 (1:25-1:55) Three icons struck through: ringing phone, hourglass, exclamation mark.
5 (1:55-2:15) Presenter, closer framing.
6 (2:15-2:30) Theatre doors open, list running. Title card "ORM - UNTH Ituku-Ozalla".
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, warm, measured, about 130 wpm. Soft instrumental under scenes 1, 2 and 6 only; silence elsewhere.

## V2 · Getting onto the theatre Wi-Fi

All staff | 3 min | Before anything else

### THE BRIEF

Nobody uses the app if they cannot get onto the network. This video is the single most-watched thing in the series, because every new member of staff needs it on their first morning. It must be slow, literal, and show a real phone.

### THEY WILL BE ABLE TO

- Join the theatre Wi-Fi and get through the sign-in page unaided.
- Understand that they sign in with their **own ORM username and password** — there is no shared Wi-Fi key.
- Stay signed in for a full 24-hour shift, and walk between theatres without signing in again.
- Know what to do when the sign-in page does not appear.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A registrar standing in a corridor, jabbing at her phone, frowning. She walks ten paces, tries again. Signal bars are full but nothing loads. | *Full signal. Nothing loading. She has been trying for four minutes, and the case she needs to book is starting in twenty. This is the most common problem in the whole hospital, and it takes ninety seconds to solve for good.* |
| 2 · 0:18–0:50<br>Join | Phone screen, 200% zoom. Settings → Wi-Fi. The theatre network appears in the list. Finger taps it. The phone connects; a small "Sign in to network" notification slides down. Finger taps the notification. | *Open your Wi-Fi settings and pick the theatre network. There is no password on the network itself, so it will connect straight away. Now watch for this: a notification that says sign in to network. Tap it. On most phones it appears on its own within a few seconds. That notification is the door.* |
| 3 · 0:50–1:25<br>The portal | The captive portal page fills the screen: hospital name, a username field, a password field, one blue Log In button. Finger types a username, then a password shown as dots. Taps Log In. A brief spinner, then a success page. | *This is the captive portal. It is not a separate account. Type the same username and password you use for the ORM itself — your own name, your own password. That is deliberate. There is no shared Wi-Fi key in this hospital, because a shared key is a key that ends up written on a wall, and then anybody in the car park is on the theatre network. Your login is yours. Tap Log In, and you are through.* |
| 4 · 1:25–1:55<br>It lasts a shift | Clock graphic sweeping 24 hours. Phone in a pocket; the screen locks; hours pass; screen unlocks and the app opens instantly with no portal. | *Now the part that matters. Once you are in, you stay in for twenty-four hours. Your phone can lock, go in your pocket, run out of battery and come back — you do not sign in again. It used to be different. One of our anaesthetists signed in fourteen times in three days. That is fixed. One sign-in covers a full shift and the night after it.* |
| 5 · 1:55–2:25<br>Roaming | Overhead animation of the suite: a router icon, then several access points along the corridor joined by a line. A staff avatar walks from Theatre 1 to Theatre 4 and to recovery; the connection indicator stays solid green throughout. | *The signal comes from one router, and the access points along the corridors extend it. To your phone it is all one network. So you can walk from Theatre One to recovery, from recovery to the store, and the app keeps working. Your device is remembered for the day, so moving between theatres never puts the sign-in page back in front of you.* |

| | | |
|---|---|---|
| 6 · 2:25–2:50<br>If it fails | Three numbered cards appear in turn, each with a small illustration. | *If the sign-in page never appears, three things fix almost every case. One: turn Wi-Fi off and on again, and wait five seconds. Two: open your browser and go to any plain address that is not secure — the portal cannot interrupt a secure page, which is why typing a search sometimes does nothing. Three: switch off mobile data while you sign in, because your phone will quietly prefer the mobile network and never ask the portal for the door. If none of those work, it is not you. Tell the theatre manager.* |
| 7 · 2:50–3:00<br>Close | The registrar from scene 1, now walking and booking on her phone, unhurried. | *One thing to do tomorrow: sign in once, at the start of your shift, before you need it. Then forget about it for the rest of the day.* |

**PRODUCTION NOTE**

Film the portal page on a **real phone**, in portrait, with a real thumb. Every previous attempt to train this with a laptop screenshot failed, because the notification behaves differently on a phone and the notification is the part people miss.

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — A registrar standing in a corridor, jabbing at her phone, frowning. She walks ten paces, tries again. Signal bars are full but nothing loads.
2. **Shot 2** (hold) — Phone screen, 200% zoom. Settings → Wi-Fi. The theatre network appears in the list. Finger taps it. The phone connects; a small "Sign in to network" notification slides down. Finger taps the notification.
3. **Shot 3** (hold) — The captive portal page fills the screen: hospital name, a username field, a password field, one blue Log In button. Finger types a username, then a password shown as dots. Taps Log In. A brief spinner, then a success page.
4. **Shot 4** (hold) — Clock graphic sweeping 24 hours. Phone in a pocket; the screen locks; hours pass; screen unlocks and the app opens instantly with no portal.
5. **Shot 5** (pan right) — Overhead animation of the suite: a router icon, then several access points along the corridor joined by a line. A staff avatar walks from Theatre 1 to Theatre 4 and to recovery; the connection indicator stays solid green throughout.
6. **Shot 6** (hold) — Three numbered cards appear in turn, each with a small illustration.
7. **Shot 7** (hold, worth animating) — The registrar from scene 1, now walking and booking on her phone, unhurried.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 7 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Full signal. Nothing loading. She has been trying for four minutes, and the case she needs to book is starting in twenty. This is the most common problem in the whole hospital, and it takes ninety seconds to solve for good.

SHOT 2 — hold
Open your Wi-Fi settings and pick the theatre network. There is no password on the network itself, so it will connect straight away. Now watch for this: a notification that says sign in to network. Tap it. On most phones it appears on its own within a few seconds. That notification is the door.

SHOT 3 — hold
This is the captive portal. It is not a separate account. Type the same username and password you use for the ORM itself — your own name, your own password. That is deliberate. There is no shared Wi-Fi key in this hospital, because a shared key is a key that ends up written on a wall, and then anybody in the car park is on the theatre network. Your login is yours. Tap Log In, and you are through.

SHOT 4 — hold
Now the part that matters. Once you are in, you stay in for twenty-four hours. Your phone can lock, go in your pocket, run out of battery and come back — you do not sign in again. It used to be different. One of our anaesthetists signed in fourteen times in three days. That is fixed. One sign-in covers a full shift and the night after it.

SHOT 5 — pan right
The signal comes from one router, and the access points along the corridors extend it. To your phone it is all one network. So you can walk from Theatre One to recovery, from recovery to the store, and the app keeps working. Your device is remembered for the day, so moving between theatres never puts the sign-in page back in front of you.

SHOT 6 — hold
If the sign-in page never appears, three things fix almost every case. One: turn Wi-Fi off and on again, and wait five seconds. Two: open your browser and go to any plain address that is not secure — the portal cannot interrupt a secure page, which is why typing a search
```

sometimes does nothing. Three: switch off mobile data while you sign in, because your phone will quietly prefer the mobile network and never ask the portal for the door. If none of those work, it is not you. Tell the theatre manager.

SHOT 7 — hold
One thing to do tomorrow: sign in once, at the start of your shift, before you need it. Then forget about it for the rest of the day.

VIDEO 2 - GETTING ONTO THE THEATRE WI-FI (3 min)
STYLE: Nigerian teaching hospital corridors. Real phone in portrait, real thumb, 200% screen zoom, tap targets highlighted a beat early.
SCENES:
1 (0:00-0:18) Registrar in a corridor jabbing at her phone, full signal, nothing loading. She walks ten paces and tries again.
2 (0:18-0:50) Phone Wi-Fi settings; theatre network selected; connects; "Sign in to network" notification slides down; thumb taps it.
3 (0:50-1:25) Captive portal page: hospital name, username, password, one blue Log In button. Types credentials, taps Log In, brief spinner, success page.
4 (1:25-1:55) 24-hour clock sweep. Phone locks, goes in a pocket, hours pass, unlocks, app opens instantly with no portal.
5 (1:55-2:25) Overhead animation of the theatre suite: router plus access points along corridors, joined by cable. Staff avatar walks Theatre 1 to recovery to store; signal indicator stays solid green.
6 (2:25-2:50) Three numbered troubleshooting cards appearing in turn.
7 (2:50-3:00) The registrar from scene 1, now booking on her phone while walking, unhurried.
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, patient and literal. This is a first-morning video; do not rush it.

## V3 · Why the network is controlled

All staff | 2 min 30 s | Pairs with V2

### THE BRIEF

Staff will discover that the theatre Wi-Fi is not an open internet cafe, and some will be annoyed. Better they hear the reason from us, framed around the patient, than invent a worse explanation. The tone is respectful and adult throughout — explaining, never scolding.

### THEY WILL BE ABLE TO

- Explain why the network is metered and monitored, in terms of clinical priority.
- Understand that their own account is tied to their activity, and what that means.
- Know what the network is for, and what to use their own mobile data for.
- Know that shared handsets do not mean shared accounts.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | Split screen. Left: an emergency booking screen spinning, trying to save. Right: a phone in a side room streaming a football match at full quality. | *On the left, an emergency case is trying to reach the theatre. On the right, a match is streaming in the changing room. They are sharing one connection. Only one of them can win, and the hospital has to decide which.* |
| 2 · 0:20–0:55<br>The reason | Presenter to camera. Behind, a bandwidth gauge animates: a wide green band labelled Clinical, a narrow band labelled Everything else. | *So the theatre network is controlled. Not to police you, and not to save money. It is controlled because an emergency booking, a blood request and a theatre alert have to get through at the exact moment somebody needs them, and a connection that everybody is streaming on cannot promise that. Clinical traffic comes first. Everything else shares what is left.* |
| 3 · 0:55–1:25<br>What it is for | Two columns build up. Left, ticked: the ORM, clinical email, hospital systems, looking something up for a patient. Right, crossed: video streaming, large downloads, file sharing, gaming. | *Here is the honest line. The theatre Wi-Fi is for work. The ORM, hospital systems, looking something up because a patient in front of you needs it — all of that, freely, and nobody will ever question it. Streaming, big downloads, sharing large files: use your own data for those. Not because they are sinful. Because they are the difference between an emergency alert arriving in one second and arriving in forty.* |
| 4 · 1:25–1:55<br>Your account | A simple log view: name, device, time signed in, time signed out. Deliberately unremarkable and boring-looking. | *Because you sign in as yourself, the network knows which account is connected and when. That is worth being plain about. It is not there to read your messages and nobody is watching your browsing. It exists so that when the network is struggling, somebody can see what is consuming it and fix it, and so that a device that should not be on a clinical network can be removed. It is the same reason theatre doors have locks.* |
| 5 · 1:55–2:20<br>Shared phones | Two nurses at a workstation, one handing a shared ward handset to the other. On screen, one signs out and the next signs in. | *One more thing, because it happens here every day. Handsets get shared. That is fine — the phone can be shared. The account cannot. If you pick up a ward phone somebody else signed in on, sign out and sign in as yourself. Otherwise* |

| | | |
|---|---|---|
| | | *your pre-operative visit gets recorded under their name, and when somebody asks who cleared that patient, the app will give the wrong answer.* |
| 6 · 2:20–2:30<br>Close | Presenter, warm. | *Use it for the patient and use it as much as you like. That is what it is for. One thing to do tomorrow: check whose name is at the top of the shared handset before you type anything into it.* |

**TONE CHECK BEFORE YOU RENDER**

Read scene 4 aloud. If it sounds like a warning, rewrite it. Staff who feel watched stop recording things, and a theatre system with missing records is more dangerous than a slow one.

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — Split screen. Left: an emergency booking screen spinning, trying to save. Right: a phone in a side room streaming a football match at full quality.
2. **Shot 2** (pan right, worth animating) — Presenter to camera. Behind, a bandwidth gauge animates: a wide green band labelled Clinical, a narrow band labelled Everything else.
3. **Shot 3** (pan right, worth animating) — Two columns build up. Left, ticked: the ORM, clinical email, hospital systems, looking something up for a patient. Right, crossed: video streaming, large downloads, file sharing, gaming.
4. **Shot 4** (push in) — A simple log view: name, device, time signed in, time signed out. Deliberately unremarkable and boring-looking.
5. **Shot 5** (hold) — Two nurses at a workstation, one handing a shared ward handset to the other. On screen, one signs out and the next signs in.
6. **Shot 6** (pull back, worth animating) — Presenter, warm.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
On the left, an emergency case is trying to reach the theatre. On the right, a match is
streaming in the changing room. They are sharing one connection. Only one of them can win,
and the hospital has to decide which.

SHOT 2 — pan right
So the theatre network is controlled. Not to police you, and not to save money. It is
controlled because an emergency booking, a blood request and a theatre alert have to get
through at the exact moment somebody needs them, and a connection that everybody is streaming
on cannot promise that. Clinical traffic comes first. Everything else shares what is left.

SHOT 3 — pan right
Here is the honest line. The theatre Wi-Fi is for work. The ORM, hospital systems, looking
something up because a patient in front of you needs it — all of that, freely, and nobody
will ever question it. Streaming, big downloads, sharing large files: use your own data for
those. Not because they are sinful. Because they are the difference between an emergency
alert arriving in one second and arriving in forty.

SHOT 4 — push in
Because you sign in as yourself, the network knows which account is connected and when. That
is worth being plain about. It is not there to read your messages and nobody is watching your
browsing. It exists so that when the network is struggling, somebody can see what is
consuming it and fix it, and so that a device that should not be on a clinical network can be
removed. It is the same reason theatre doors have locks.

SHOT 5 — hold
One more thing, because it happens here every day. Handsets get shared. That is fine — the
phone can be shared. The account cannot. If you pick up a ward phone somebody else signed in
on, sign out and sign in as yourself. Otherwise your pre-operative visit gets recorded under
their name, and when somebody asks who cleared that patient, the app will give the wrong
answer.

SHOT 6 — pull back
Use it for the patient and use it as much as you like. That is what it is for. One thing to
```

do tomorrow: check whose name is at the top of the shared handset before you type anything into it.

VIDEO 3 - WHY THE NETWORK IS CONTROLLED (2 min 30 s)
STYLE: Nigerian teaching hospital. Respectful, adult, explanatory. Never scolding.
SCENES:
1 (0:00-0:20) Split screen: left, an emergency booking screen spinning while trying to save; right, a phone streaming football at full quality in a changing room.
2 (0:20-0:55) Presenter to camera; behind, a bandwidth gauge with a wide green "Clinical" band and a narrow "Everything else" band.
3 (0:55-1:25) Two columns building: ticked - ORM, clinical email, hospital systems, looking something up for a patient; crossed - streaming, large downloads, file sharing, gaming.
4 (1:25-1:55) A plain, boring log view: name, device, sign-in time, sign-out time.
5 (1:55-2:20) Two nurses at a workstation sharing a ward handset; on screen one account signs out and another signs in.
6 (2:20-2:30) Presenter, warm close.
NARRATION: paste the narration column of the scene table, in order.
VOICE: Nigerian English, calm and respectful. Scene 4 must sound like an explanation, not a warning.

## V4 · What follows you everywhere

All staff | 3 min

### THE BRIEF

Five behaviours exist on every screen in the app. Teaching them once here saves explaining them in thirty-four later videos.

### THEY WILL BE ABLE TO

- Understand the theatre radio, and why it ducks the music instead of stopping it.
- Trust that a half-finished form survives switching apps.
- Keep working when the internet drops, and know what happens to what they typed.
- Find the Feedback button and the Role Guide.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A nurse half-way through a long pre-op form. Her phone rings; she takes the call, opens WhatsApp to check a folder number, comes back eight minutes later. The form is exactly as she left it. | *She left this form for eight minutes. A call, then WhatsApp, then back. Watch what did not happen. Nothing disappeared. That used to be the single most hated thing about this app, and it is now the first thing you should know about it.* |
| 2 · 0:20–0:55 Drafts | Same form. A small grey line under the header reads "Saved 09:12". Phone is locked, reopened; a soft banner says the entry was restored. | *Every long form keeps a private draft on your own phone as you type. Switch apps, lock the screen, let the battery die — when you come back it offers your work back to you. Two rules protect you. It is kept for a few hours only, so yesterday's half-typed case never reappears by accident. And it is tied to the person who typed it — on a shared ward handset, your draft is never handed to the next person who signs in.* |
| 3 · 0:55–1:35 The radio | Speaker icon in a corridor. Soft music playing, waveform visible. An announcement begins; the music waveform drops low but does not stop; the announcement finishes and the music rises again. | *The theatre radio speaks announcements out loud: send for the patient, the patient has arrived, an emergency has been booked. If music is playing, it does not stop — it drops low underneath and comes back afterwards. That is deliberate. Music cutting dead in a theatre is itself a distraction, and people look up at the wrong moment. Only one screen speaks, even when the app is open on three computers at the same desk, so you hear an announcement once and not three times.* |
| 4 · 1:35–2:15 Offline | Phone with the Wi-Fi symbol crossed out. The user saves a milestone; a small "queued" marker appears. Connection returns; the marker clears with a tick. | *The signal in this hospital is not perfect and the app is built for that. If you record something while the connection is down, it is held on your phone and sent the moment you are back — you do not have to remember it or type it twice. Two things to know. It waits about thirty seconds before telling you it could not reach the server, so give it that time before pressing again. And if you do press twice, the app recognises the repeat and will not book the same case two times.* |
| 5 · 2:15–2:45 Help | Sidebar scrolls to the bottom: Role Guide, Staff Training, Feedback. | *Three things live at the bottom of the menu and almost nobody finds them. Role Guide tells you what your own role* |

| | | |
|---|---|---|
| | | *is expected to do, in plain words. Staff Training holds these videos. Feedback goes straight to the people who build this. Use it. It is the shortest route between something annoying you and something changing.* |
| 6 · 2:45–3:00<br>Close | Presenter. | *One thing to do tomorrow: start a form, deliberately switch to another app, and come back. Once you have seen it survive, you will stop rushing, and rushing is what causes the mistakes.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A nurse half-way through a long pre-op form. Her phone rings; she takes the call, opens WhatsApp to check a folder number, comes back eight minutes later. The form is exactly as she left it.
2. **Shot 2** (hold) — Same form. A small grey line under the header reads "Saved 09:12". Phone is locked, reopened; a soft banner says the entry was restored.
3. **Shot 3** (hold) — Speaker icon in a corridor. Soft music playing, waveform visible. An announcement begins; the music waveform drops low but does not stop; the announcement finishes and the music rises again.
4. **Shot 4** (hold) — Phone with the Wi-Fi symbol crossed out. The user saves a milestone; a small "queued" marker appears. Connection returns; the marker clears with a tick.
5. **Shot 5** (push in, worth animating) — Sidebar scrolls to the bottom: Role Guide, Staff Training, Feedback.
6. **Shot 6** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
She left this form for eight minutes. A call, then WhatsApp, then back. Watch what did not happen. Nothing disappeared. That used to be the single most hated thing about this app, and it is now the first thing you should know about it.

SHOT 2 — hold
Every long form keeps a private draft on your own phone as you type. Switch apps, lock the screen, let the battery die — when you come back it offers your work back to you. Two rules protect you. It is kept for a few hours only, so yesterday's half-typed case never reappears by accident. And it is tied to the person who typed it — on a shared ward handset, your draft is never handed to the next person who signs in.

SHOT 3 — hold
The theatre radio speaks announcements out loud: send for the patient, the patient has arrived, an emergency has been booked. If music is playing, it does not stop — it drops low underneath and comes back afterwards. That is deliberate. Music cutting dead in a theatre is itself a distraction, and people look up at the wrong moment. Only one screen speaks, even when the app is open on three computers at the same desk, so you hear an announcement once and not three times.

SHOT 4 — hold
The signal in this hospital is not perfect and the app is built for that. If you record something while the connection is down, it is held on your phone and sent the moment you are back — you do not have to remember it or type it twice. Two things to know. It waits about thirty seconds before telling you it could not reach the server, so give it that time before pressing again. And if you do press twice, the app recognises the repeat and will not book the same case two times.

SHOT 5 — push in
Three things live at the bottom of the menu and almost nobody finds them. Role Guide tells you what your own role is expected to do, in plain words. Staff Training holds these videos. Feedback goes straight to the people who build this. Use it. It is the shortest route between something annoying you and something changing.

SHOT 6 — pull back
One thing to do tomorrow: start a form, deliberately switch to another app, and come back.
```

> Once you have seen it survive, you will stop rushing, and rushing is what causes the mistakes.

VIDEO 4 - WHAT FOLLOWS YOU EVERYWHERE (3 min)
SCENES: 1 (0:00-0:20) Nurse mid-form, takes a call, opens WhatsApp, returns after 8 minutes; form intact. 2 (0:20-0:55) Same form with a small "Saved 09:12" line; phone locked and reopened; soft "entry restored" banner. 3 (0:55-1:35) Corridor speaker; music waveform ducks low under an announcement, then rises. 4 (1:35-2:15) Phone with Wi-Fi crossed out; a milestone saved shows "queued"; connection returns and it clears with a tick. 5 (2:15-2:45) Sidebar scrolling to Role Guide, Staff Training, Feedback. 6 (2:45-3:00) Presenter close.
NARRATION: paste the narration column, in order.
VOICE: Nigerian English, reassuring.

## Part 2 — Surgical team

Surgeons, consultant surgeons and house officers. The list begins here: almost everything the rest of the hospital does tomorrow is decided by what is typed into these screens today.

## V5 · Booking an elective case

Surgeon | Consultant surgeon | House officer | 4 min

### THE BRIEF

The most important screen in the application. Every downstream department is triggered by this one form, and the commonest cause of a cancelled case is something left blank here. Teach it as a chain of consequences, not a list of fields.

### THEY WILL BE ABLE TO

- Book a complete elective case, including packs and drugs, in one sitting.
- Explain why start time and expected duration are compulsory.
- Give the patient their pack codes before they leave the clinic.
- Recognise the outstanding-items banner and act on it the same day.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A cancelled case. Patient sitting on a trolley in a corridor, gown on, being wheeled back. Caption: "Cancelled — mesh not available". | *This patient fasted from midnight. She was cancelled at ten past nine because the mesh was not in the pack. Nobody did anything wrong on the morning. It was decided three weeks earlier, on the booking form, in about four seconds.* |
| 2 · 0:20–0:55 Patient | Laptop. Surgeries → Book Surgery. Search field; typing a folder number brings up an existing patient. Then a demo of registering a new patient: name, folder number, age, sex, ward. | *Start in Surgeries, then Book Surgery. Search by folder number first — most patients are already in the system, and finding them keeps their whole history attached to this case. If it is a new patient, register them here: name, folder number, age, sex and ward. Take care with the folder number. It is the one thing the pharmacy, the blood bank and the ward all use to find her.* |
| 3 · 0:55–1:35 The operation | Unit dropdown, then procedure with type-to-search. Indication field. Magnitude set to Major. Anaesthesia set to General. Date picker, then start time 09:00 and duration 240 minutes, both highlighted. | *Now the operation. Choose your unit, then the procedure — type two or three letters and it finds it. Write the indication as you would in the notes. Then two fields that people try to skip, and cannot. Start time, and how long you expect it to take. Both are compulsory, and here is why: a case with no start time cannot be judged late, and a case with no duration means the next case on that list cannot be scheduled after it. Guess honestly rather than leaving it. An honest four hours is far more useful than a hopeful ninety minutes.* |
| 4 · 1:35–2:20 Packs | The consumables list auto-fills from the chosen procedure. Surgeon adds two extra items. Drug and dressing list beside it. Special needs toggles: blood transfusion, diathermy, C-arm. | *This is where the mesh was lost. When you pick the procedure, the app fills in the usual pack for it, and adds the standard theatre pack on top. Read it. If this patient needs something extra, add it now — you are the only person in the building who knows that. The moment you save, this list becomes a job on the pack provider's phone and the drug list becomes a job for pharmacy, tonight, while the store is still open. Tick the special needs too. Blood transfusion here is what tells the blood bank to expect a request.* |
| 5 · 2:20–2:55 Safety | Pre-operative section: haemoglobin, potassium, sodium, creatinine, HBsAg, HCV, HIV, blood pressure. | *The pre-operative results. Fill in whatever you have. What you do not have, leave — the case will still book. That was a deliberate change: cases were being blocked at booking* |

| | | |
|---|---|---|
| | Some filled, some empty. Consent section with an upload control. | *because a resident did not yet hold a result somebody else was carrying. What is missing does not disappear. It comes back as an outstanding item on the case, visible to everybody, until whoever holds it enters it.* |
| 6 · 2:55–3:30<br>Save | Save pressed. Confirmation panel showing the case, an amber banner reading "CONSENT + 6 PRE-OP ITEMS OUTSTANDING", and two codes labelled consumable pack code and pharmacy code. | *Save. Three things now exist. The case is on tomorrow's list and everybody who needs to see it can. The amber banner tells you exactly what is still missing — treat that as your job list for today, not somebody else's for the morning. And these two codes are for the patient: write them on her card. She hands them in at the store and at the pharmacy, and they find her pack without a single phone call.* |
| 7 · 3:30–4:00<br>Close | The corridor from scene 1, but the trolley goes forward into theatre. Presenter. | *Booking is not paperwork. It is the instruction the whole hospital follows tomorrow. One thing to do on your next list: before you close the form, read the amber banner out loud. If it mentions consent, fix it today.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A cancelled case. Patient sitting on a trolley in a corridor, gown on, being wheeled back. Caption: "Cancelled — mesh not available".
2. **Shot 2** (hold, worth animating) — Laptop. Surgeries → Book Surgery. Search field; typing a folder number brings up an existing patient. Then a demo of registering a new patient: name, folder number, age, sex, ward.
3. **Shot 3** (hold) — Unit dropdown, then procedure with type-to-search. Indication field. Magnitude set to Major. Anaesthesia set to General. Date picker, then start time 09:00 and duration 240 minutes, both highlighted.
4. **Shot 4** (hold, worth animating) — The consumables list auto-fills from the chosen procedure. Surgeon adds two extra items. Drug and dressing list beside it. Special needs toggles: blood transfusion, diathermy, C-arm.
5. **Shot 5** (push in) — Pre-operative section: haemoglobin, potassium, sodium, creatinine, HBsAg, HCV, HIV, blood pressure. Some filled, some empty. Consent section with an upload control.
6. **Shot 6** (push in) — Save pressed. Confirmation panel showing the case, an amber banner reading "CONSENT + 6 PRE-OP ITEMS OUTSTANDING", and two codes labelled consumable pack code and pharmacy code.
7. **Shot 7** (pan left, worth animating) — The corridor from scene 1, but the trolley goes forward into theatre. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 7 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
This patient fasted from midnight. She was cancelled at ten past nine because the mesh was
not in the pack. Nobody did anything wrong on the morning. It was decided three weeks
earlier, on the booking form, in about four seconds.

SHOT 2 — hold
Start in Surgeries, then Book Surgery. Search by folder number first — most patients are
already in the system, and finding them keeps their whole history attached to this case. If
it is a new patient, register them here: name, folder number, age, sex and ward. Take care
with the folder number. It is the one thing the pharmacy, the blood bank and the ward all use
to find her.

SHOT 3 — hold
Now the operation. Choose your unit, then the procedure — type two or three letters and it
finds it. Write the indication as you would in the notes. Then two fields that people try to
skip, and cannot. Start time, and how long you expect it to take. Both are compulsory, and
here is why: a case with no start time cannot be judged late, and a case with no duration
means the next case on that list cannot be scheduled after it. Guess honestly rather than
leaving it. An honest four hours is far more useful than a hopeful ninety minutes.

SHOT 4 — hold
This is where the mesh was lost. When you pick the procedure, the app fills in the usual pack
for it, and adds the standard theatre pack on top. Read it. If this patient needs something
extra, add it now — you are the only person in the building who knows that. The moment you
save, this list becomes a job on the pack provider's phone and the drug list becomes a job
for pharmacy, tonight, while the store is still open. Tick the special needs too. Blood
transfusion here is what tells the blood bank to expect a request.

SHOT 5 — push in
The pre-operative results. Fill in whatever you have. What you do not have, leave — the case
will still book. That was a deliberate change: cases were being blocked at booking because a
resident did not yet hold a result somebody else was carrying. What is missing does not
disappear. It comes back as an outstanding item on the case, visible to everybody, until
whoever holds it enters it.
```

SHOT 6 — push in
Save. Three things now exist. The case is on tomorrow's list and everybody who needs to see it can. The amber banner tells you exactly what is still missing — treat that as your job list for today, not somebody else's for the morning. And these two codes are for the patient: write them on her card. She hands them in at the store and at the pharmacy, and they find her pack without a single phone call.

SHOT 7 — pan left
Booking is not paperwork. It is the instruction the whole hospital follows tomorrow. One thing to do on your next list: before you close the form, read the amber banner out loud. If it mentions consent, fix it today.

VIDEO 5 - BOOKING AN ELECTIVE CASE (4 min)
STYLE: laptop screen capture at 150%, tap targets highlighted a beat early; intercut with a Nigerian theatre corridor.
SCENES: 1 (0:00-0:20) A gowned patient wheeled back from theatre, caption "Cancelled - mesh not available". 2 (0:20-0:55) Surgeries > Book Surgery; search by folder number; registering a new patient. 3 (0:55-1:35) Unit, procedure type-to-search, indication, magnitude Major, anaesthesia General, date, start time 09:00, duration 240 - the last two highlighted. 4 (1:35-2:20) Consumables auto-filling from the procedure; two extra items added; drug list; special-needs toggles. 5 (2:20-2:55) Pre-op labs partly filled; consent upload control. 6 (2:55-3:30) Save; confirmation with amber banner "CONSENT + 6 PRE-OP ITEMS OUTSTANDING" and two patient codes. 7 (3:30-4:00) Trolley now going forward into theatre; presenter close.
NARRATION: paste the narration column, in order.

## V6 · Emergency booking

Surgeon | House officer | Anaesthetist | 3 min 30 s

### THE BRIEF

Booked under pressure, often one-handed, often at night. The form is a stepped wizard with a progress bar for exactly that reason. Emphasise that it can be finished in stages and that the theatre hears about it immediately.

### THEY WILL BE ABLE TO

- Raise an emergency case in under two minutes and know the theatre has heard.
- Work through all the steps without the form closing early.
- Upload a signed consent rather than re-typing one.
- Understand why an emergency cannot be dated in the past.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | Accident and emergency at night. A registrar with a phone in one hand, gloved. Urgency without chaos. | *Ten at night. Perforated bowel. He has one free hand and about ninety seconds before he is needed again. This form is built for exactly this moment, and it is built so that ninety seconds is enough.* |
| 2 · 0:20–0:55 Step 1 | Phone. Emergency Booking. A progress bar across the top showing five steps, step 1 filled. Patient details, unit, procedure, indication, priority, required-by time. | *Emergency Booking, first item in the menu. Notice the bar across the top: five steps, and you can see where you are at all times. Step one is the patient and the operation. Priority, and the time the surgery is needed by. That time cannot be in the past — the app will refuse it. It sounds obvious, but a mistyped date once put a case a week behind us, and the theatre heard the alert for a patient no screen could show.* |
| 3 · 0:55–1:35 Sent | Save and send. On screen, a confirmation. Cut to the theatre: a speaker, and the emergency board lighting up with the patient. Cut to phones buzzing in three pockets. | *Save and send. Right now, before you have typed anything else, the theatre knows. It is announced on the radio, it appears on the emergency board, and the team's phones alert — loudly, even if the phone is locked and on silent, because that is what an emergency alert is for. Now — and this is the part people used to get wrong — the form does not close. Keep going.* |
| 4 · 1:35–2:20 Packs then consent | Step: consumables and drugs. Then the next step: a single upload control for a signed consent form, with a camera option. A photograph of a signed paper consent is taken and attached. | *Packs come next, and deliberately before consent. The store needs the longest warning, so it is asked for first. Then consent, and this step is only one thing: upload the signed form. Photograph the paper. Do not re-type it, do not summarise it — the signed sheet is the record, and a photograph of it is the fastest honest way to get it into the system at ten at night.* |
| 5 · 2:20–2:55 Team | Team step: surgeon, assistant, anaesthetist. Then the completion screen with the case summary. | *Last, the team. Add who you know; leave what you do not. The case is already live, so this is refinement, not permission. And if you are called away half-way through, what you typed is kept — come back and finish it. An emergency booked in three visits to the form is still infinitely better than one held in your head.* |

| | | |
|---|---|---|
| 6 · 2:55–3:30<br>Close | The emergency board on a wall screen, the case sitting on it with its priority colour. Presenter. | *The moment you press save on step one, you have stopped being the only person who knows. One thing to remember: finish the remaining steps the same night, even from the corridor. The store cannot pack what it has not been told about.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — Accident and emergency at night. A registrar with a phone in one hand, gloved. Urgency without chaos.
2. **Shot 2** (hold, worth animating) — Phone. Emergency Booking. A progress bar across the top showing five steps, step 1 filled. Patient details, unit, procedure, indication, priority, required-by time.
3. **Shot 3** (hold) — Save and send. On screen, a confirmation. Cut to the theatre: a speaker, and the emergency board lighting up with the patient. Cut to phones buzzing in three pockets.
4. **Shot 4** (hold) — Step: consumables and drugs. Then the next step: a single upload control for a signed consent form, with a camera option. A photograph of a signed paper consent is taken and attached.
5. **Shot 5** (hold) — Team step: surgeon, assistant, anaesthetist. Then the completion screen with the case summary.
6. **Shot 6** (hold) — The emergency board on a wall screen, the case sitting on it with its priority colour. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Ten at night. Perforated bowel. He has one free hand and about ninety seconds before he is needed again. This form is built for exactly this moment, and it is built so that ninety seconds is enough.

SHOT 2 — hold
Emergency Booking, first item in the menu. Notice the bar across the top: five steps, and you can see where you are at all times. Step one is the patient and the operation. Priority, and the time the surgery is needed by. That time cannot be in the past — the app will refuse it. It sounds obvious, but a mistyped date once put a case a week behind us, and the theatre heard the alert for a patient no screen could show.

SHOT 3 — hold
Save and send. Right now, before you have typed anything else, the theatre knows. It is announced on the radio, it appears on the emergency board, and the team's phones alert — loudly, even if the phone is locked and on silent, because that is what an emergency alert is for. Now — and this is the part people used to get wrong — the form does not close. Keep going.

SHOT 4 — hold
Packs come next, and deliberately before consent. The store needs the longest warning, so it is asked for first. Then consent, and this step is only one thing: upload the signed form. Photograph the paper. Do not re-type it, do not summarise it — the signed sheet is the record, and a photograph of it is the fastest honest way to get it into the system at ten at night.

SHOT 5 — hold
Last, the team. Add who you know; leave what you do not. The case is already live, so this is refinement, not permission. And if you are called away half-way through, what you typed is kept — come back and finish it. An emergency booked in three visits to the form is still infinitely better than one held in your head.

SHOT 6 — hold
The moment you press save on step one, you have stopped being the only person who knows. One thing to remember: finish the remaining steps the same night, even from the corridor. The store cannot pack what it has not been told about.
```

VIDEO 6 - EMERGENCY BOOKING (3 min 30 s)
STYLE: night, A&E, phone held one-handed. Urgent but controlled.
SCENES: 1 (0:00-0:20) Registrar at night, gloved, phone in one hand. 2 (0:20-0:55) Phone: Emergency Booking, five-step progress bar, step 1 - patient, unit, procedure, indication, priority, required-by time. 3 (0:55-1:35) Save and send; cut to theatre speaker, emergency board lighting up, three phones buzzing. 4 (1:35-2:20) Packs and drugs step; then consent step - a single upload control; photographing a signed paper consent. 5 (2:20-2:55) Team step; completion summary. 6 (2:55-3:30) Wall-mounted emergency board showing the case; presenter close.
NARRATION: paste the narration column, in order.

## V7 · Consent and pre-op blocks

Surgeon | House officer | Ward nurse | 3 min

### THE BRIEF

Consent is the single commonest reason a case is held in the morning. Two different people can settle it, and most staff know only one of the routes.

### THEY WILL BE ABLE TO

- Record consent as a surgeon, and clear the morning block.
- Record consent as a nurse during the pre-operative visit, and know that this also settles it.
- Read the outstanding-items banner and tell which items block calling the patient.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | 08:05. A theatre nurse at a screen, phone to ear, patient not sent for. Case card shows a red "Consent outstanding". | *Five past eight. The theatre is ready and the patient has not been sent for, because the app says the consent is outstanding. It was signed last night. Nobody told the app.* |
| 2 · 0:18–1:00<br>Surgeon route | Case → Consent. Upload the signed form, or record an electronic signature. Save. The red banner turns green. | *If you are the surgeon, open the case and go to Consent. Two ways in: upload a photograph of the signed paper, or record it signed electronically. Either satisfies it. Save, and the block clears immediately — the morning team can call for the patient without ringing anybody. Consultants and registrars both have this. If you were told you did not have permission, that was a fault, and it is fixed.* |
| 3 · 1:00–1:45<br>Nurse route | Pre-Operative Visit form on a phone at the bedside. A checkbox: consent present and signed. Ticked. Save. | *And if you are the nurse doing the pre-operative visit the night before, you can settle it too. When you confirm on the visit form that the consent is present and signed, the app takes that as settled. You are standing at the bedside holding the folder — you are a better witness to that than anybody at a desk. This is the route most people do not know exists, and it is the one that prevents the eight o'clock phone call.* |
| 4 · 1:45–2:25<br>Refusal | The same tick box set to "refused". The case shows a firm red state that does not clear. | *One exception, and it is absolute. If consent has been refused, nothing overrides it. Not a later upload, not a tick, not a manager. A refusal stands until the patient themselves changes it. The app is built that way on purpose, and you should expect it to argue with you.* |
| 5 · 2:25–2:50<br>The other items | Banner expanded showing the outstanding list: haemoglobin, HIV, HBsAg, blood pressure, packs, drugs. Some marked as blocking, others informational. | *Consent is one of several outstanding items. The others — the bloods, the pack list, the drugs — do not stop the patient being called, but every one of them is somebody's job today. Open the banner. Whatever is on it is a phone call that will otherwise happen tomorrow morning at the worst possible time.* |
| 6 · 2:50–3:00<br>Close | The nurse from scene 1, now sending for the patient calmly. | *One thing to do tonight: on your pre-operative round, tick the consent box while the folder is in your hand.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — 08:05. A theatre nurse at a screen, phone to ear, patient not sent for. Case card shows a red "Consent outstanding".
2. **Shot 2** (hold) — Case → Consent. Upload the signed form, or record an electronic signature. Save. The red banner turns green.
3. **Shot 3** (hold) — Pre-Operative Visit form on a phone at the bedside. A checkbox: consent present and signed. Ticked. Save.
4. **Shot 4** (push in) — The same tick box set to "refused". The case shows a firm red state that does not clear.
5. **Shot 5** (hold) — Banner expanded showing the outstanding list: haemoglobin, HIV, HBsAg, blood pressure, packs, drugs. Some marked as blocking, others informational.
6. **Shot 6** (pull back, worth animating) — The nurse from scene 1, now sending for the patient calmly.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Five past eight. The theatre is ready and the patient has not been sent for, because the app
says the consent is outstanding. It was signed last night. Nobody told the app.

SHOT 2 — hold
If you are the surgeon, open the case and go to Consent. Two ways in: upload a photograph of
the signed paper, or record it signed electronically. Either satisfies it. Save, and the
block clears immediately — the morning team can call for the patient without ringing anybody.
Consultants and registrars both have this. If you were told you did not have permission, that
was a fault, and it is fixed.

SHOT 3 — hold
And if you are the nurse doing the pre-operative visit the night before, you can settle it
too. When you confirm on the visit form that the consent is present and signed, the app takes
that as settled. You are standing at the bedside holding the folder — you are a better
witness to that than anybody at a desk. This is the route most people do not know exists, and
it is the one that prevents the eight o'clock phone call.

SHOT 4 — push in
One exception, and it is absolute. If consent has been refused, nothing overrides it. Not a
later upload, not a tick, not a manager. A refusal stands until the patient themselves
changes it. The app is built that way on purpose, and you should expect it to argue with you.

SHOT 5 — hold
Consent is one of several outstanding items. The others — the bloods, the pack list, the
drugs — do not stop the patient being called, but every one of them is somebody's job today.
Open the banner. Whatever is on it is a phone call that will otherwise happen tomorrow
morning at the worst possible time.

SHOT 6 — pull back
One thing to do tonight: on your pre-operative round, tick the consent box while the folder
is in your hand.
```

```
VIDEO 7 - CONSENT AND PRE-OP BLOCKS (3 min)
SCENES: 1 (0:00-0:18) 08:05, theatre nurse on the phone, case card showing red "Consent
```

```
outstanding". 2 (0:18-1:00) Case > Consent; upload signed form or record electronic signature;
banner turns green. 3 (1:00-1:45) Pre-Operative Visit form on a phone at the bedside; "consent
present and signed" ticked; saved. 4 (1:45-2:25) Same control set to "refused"; a firm red state
that will not clear. 5 (2:25-2:50) Outstanding-items banner expanded: haemoglobin, HIV, HBsAg,
blood pressure, packs, drugs. 6 (2:50-3:00) The nurse from scene 1 calmly sending for the
patient.
NARRATION: paste the narration column, in order.
```

## V8 · The house officer's day

House officer | 3 min

### THE BRIEF

House officers do more data entry in this app than anyone else and receive the least training. Frame the whole video around their own board, so the app becomes a to-do list rather than a filing duty.

### THEY WILL BE ABLE TO

- Open their own board and see only their cases.
- Clear outstanding pre-op items in one pass.
- Chase the right person by WhatsApp from inside the app.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A house officer with three folders under one arm, phone in the other hand, walking fast. | *Six patients, four wards, one ward round, and every consultant assumes you are holding all of it in your head. You do not have to. The app already knows which cases are yours.* |
| 2 · 0:20–1:00<br>My board | Phone. Dashboard opens on a personal board: only this user's cases, each with its outstanding items listed underneath. | *Your dashboard shows the cases you are linked to — the ones you booked and the ones where you are on the team. Every case carries its own list of what is missing. That is your round, written for you, in the order it matters. Work down it, not across the ward.* |
| 3 · 1:00–1:50<br>Clearing items | A case opened. Haemoglobin typed in, sample time recorded. HIV and HBsAg set to negative. Each item disappears from the list as it is entered. The banner count drops from six to two. | *Tap any case and fill in what you now have. Haemoglobin and the time the sample was taken. The viral screen. The blood pressure. Each one you enter drops off the list in front of you, and the count goes down. You will feel it. That count is also what your consultant sees, so a case sitting at six outstanding items on the evening before is a conversation you would rather not have.* |
| 4 · 1:50–2:30<br>Chasing | A case where the booking was made by someone else. Card shows the booker's name, role and phone, with a "Chase on WhatsApp" button. Tapped, it opens WhatsApp with a message already written. | *Some things are not yours to fix. When a case has outstanding items and you are not the person who can settle them, the card tells you who booked it, their role and their number — and Chase on WhatsApp writes the message for you, with the patient, the operation and exactly what is missing. Fifteen seconds instead of a ten-minute hunt for a phone number.* |
| 5 · 2:30–3:00<br>Close | End of day, the board showing all items cleared. Presenter. | *Every item you clear today is a case that will not be delayed tomorrow, and everyone will assume the morning simply went well. One thing to do tomorrow: open your board before the ward round, not after it.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A house officer with three folders under one arm, phone in the other hand, walking fast.
2. **Shot 2** (hold) — Phone. Dashboard opens on a personal board: only this user's cases, each with its outstanding items listed underneath.
3. **Shot 3** (hold) — A case opened. Haemoglobin typed in, sample time recorded. HIV and HBsAg set to negative. Each item disappears from the list as it is entered. The banner count drops from six to two.
4. **Shot 4** (hold) — A case where the booking was made by someone else. Card shows the booker's name, role and phone, with a "Chase on WhatsApp" button. Tapped, it opens WhatsApp with a message already written.
5. **Shot 5** (hold) — End of day, the board showing all items cleared. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Six patients, four wards, one ward round, and every consultant assumes you are holding all of it in your head. You do not have to. The app already knows which cases are yours.

SHOT 2 — hold
Your dashboard shows the cases you are linked to — the ones you booked and the ones where you are on the team. Every case carries its own list of what is missing. That is your round, written for you, in the order it matters. Work down it, not across the ward.

SHOT 3 — hold
Tap any case and fill in what you now have. Haemoglobin and the time the sample was taken. The viral screen. The blood pressure. Each one you enter drops off the list in front of you, and the count goes down. You will feel it. That count is also what your consultant sees, so a case sitting at six outstanding items on the evening before is a conversation you would rather not have.

SHOT 4 — hold
Some things are not yours to fix. When a case has outstanding items and you are not the person who can settle them, the card tells you who booked it, their role and their number — and Chase on WhatsApp writes the message for you, with the patient, the operation and exactly what is missing. Fifteen seconds instead of a ten-minute hunt for a phone number.

SHOT 5 — hold
Every item you clear today is a case that will not be delayed tomorrow, and everyone will assume the morning simply went well. One thing to do tomorrow: open your board before the ward round, not after it.
```

```
VIDEO 8 - THE HOUSE OFFICER'S DAY (3 min)
SCENES: 1 (0:00-0:20) House officer with three folders and a phone, walking fast. 2 (0:20-1:00) Phone dashboard showing a personal board of only their cases, each with outstanding items beneath. 3 (1:00-1:50) A case opened; haemoglobin, sample time, HIV, HBsAg entered; items vanish; banner count drops 6 to 2. 4 (1:50-2:30) A case booked by someone else showing booker name, role, phone, and a "Chase on WhatsApp" button that opens a pre-written message. 5 (2:30-3:00) End of day, board clear; presenter close.
NARRATION: paste the narration column, in order.
```

## V9 · The consultant's oversight

Consultant surgeon | 3 min

### THE BRIEF

Consultants will not do data entry, and should not be asked to. Sell this entirely as oversight: seeing the firm's whole workload without asking anyone.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A consultant in an office, being handed a paper list by a registrar. The list is handwritten, amended twice. | *This list was accurate when it was written. That was yesterday afternoon. Two cases have been added since, one has been cancelled, and nobody has told him. There is a version of it that is never out of date.* |
| 2 · 0:18–1:00<br>Supervision | Surgeries list filtered by unit; cases show "Consultant: [name]" on each card. | *Every case names its supervising consultant, so filtering by your unit gives you your firm's real workload — today, tomorrow, next week. Cases your registrars booked appear the moment they book them, not when somebody remembers to tell you.* |
| 3 · 1:00–1:45<br>My Practice | My Practice screen: case counts, case mix, on-time starts, cancellations with reasons. | *My Practice is your own record: how many cases, what mix, how many started on time, how many were cancelled and why. This is not surveillance — nobody else is browsing it. It is the number you need when you are asked to justify a theatre slot, and it is built from timestamps rather than memory.* |
| 4 · 1:45–2:30<br>Governance | QA Review screen; then the Mortality Registry with an entry being opened. | *Two more, for the parts of the job nobody enjoys. QA Review holds cases flagged for discussion, with the full timeline attached — who was present, when the knife went to skin, what was delayed. And the Mortality Registry is the formal record. Both exist so that a morbidity meeting argues about the case rather than about what happened.* |
| 5 · 2:30–3:00<br>Close | Consultant closing the laptop, walking into theatre. Presenter. | *You will use four screens and never fill in a form. One thing to do this week: open My Practice and look at your cancellation reasons. Two of them will be fixable by your own team, on the booking form.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — A consultant in an office, being handed a paper list by a registrar. The list is handwritten, amended twice.
2. **Shot 2** (hold, worth animating) — Surgeries list filtered by unit; cases show "Consultant: [name]" on each card.
3. **Shot 3** (hold) — My Practice screen: case counts, case mix, on-time starts, cancellations with reasons.
4. **Shot 4** (hold) — QA Review screen; then the Mortality Registry with an entry being opened.
5. **Shot 5** (hold, worth animating) — Consultant closing the laptop, walking into theatre. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
This list was accurate when it was written. That was yesterday afternoon. Two cases have been added since, one has been cancelled, and nobody has told him. There is a version of it that is never out of date.

SHOT 2 — hold
Every case names its supervising consultant, so filtering by your unit gives you your firm's real workload — today, tomorrow, next week. Cases your registrars booked appear the moment they book them, not when somebody remembers to tell you.

SHOT 3 — hold
My Practice is your own record: how many cases, what mix, how many started on time, how many were cancelled and why. This is not surveillance — nobody else is browsing it. It is the number you need when you are asked to justify a theatre slot, and it is built from timestamps rather than memory.

SHOT 4 — hold
Two more, for the parts of the job nobody enjoys. QA Review holds cases flagged for discussion, with the full timeline attached — who was present, when the knife went to skin, what was delayed. And the Mortality Registry is the formal record. Both exist so that a morbidity meeting argues about the case rather than about what happened.

SHOT 5 — hold
You will use four screens and never fill in a form. One thing to do this week: open My Practice and look at your cancellation reasons. Two of them will be fixable by your own team, on the booking form.
```

```
VIDEO 9 - THE CONSULTANT'S OVERSIGHT (3 min)
SCENES: 1 (0:00-0:18) Consultant handed a twice-amended handwritten list. 2 (0:18-1:00) Surgeries filtered by unit; each card naming its supervising consultant. 3 (1:00-1:45) My Practice: case counts, case mix, on-time starts, cancellations with reasons. 4 (1:45-2:30) QA Review with a case timeline; then the Mortality Registry. 5 (2:30-3:00) Consultant closing the laptop and walking into theatre.
NARRATION: paste the narration column, in order.
```

## Part 3 — Anaesthesia

Anaesthetists, consultant anaesthetists and anaesthetic technicians. This part carries one rule that must be said out loud in every video: a planned case is covered by the anaesthetist rostered to its specialty, or by nobody.

## V10 · The anaesthetist board

Anaesthetist | Consultant anaesthetist | 3 min 30 s

### THEY WILL BE ABLE TO

- Open the board and see every case they are covering tomorrow.
- Complete a pre-operative anaesthetic review from the ward.
- Spot an unfit patient the evening before rather than at the theatre door.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | 08:10, theatre door. An anaesthetist meets a patient for the first time; the observation chart shows a potassium of 6.9. The case is stood down. | *He is meeting this patient for the first time at the theatre door. Her potassium is six point nine. The case is now cancelled, the list has lost two hours, and every one of those facts was available last night.* |
| 2 · 0:20–1:05 The board | Anaesthetist Board on a phone: tomorrow's cases grouped by theatre, each showing patient, procedure, anaesthesia type, and a coloured review status. | *The Anaesthetist Board is your list, not the surgeon's. Tomorrow's cases, grouped by theatre, each showing the operation, the planned anaesthetic and whether it has been reviewed. Open it the evening before. Twenty minutes with this screen is the difference between a list that runs and a morning spent apologising.* |
| 3 · 1:05–2:00 The review | A case opened. Pre-op review form: airway assessment, ASA grade, comorbidities, current medication, the lab results already entered by the team, and a fitness decision with a free-text note. | *Tap a case for the pre-operative review. The bloods the surgical team entered are already here — you are not chasing them. Add the airway assessment, the ASA grade, the comorbidities and the drugs she is on. Then the decision: fit, fit with conditions, or not fit. Write the reason. That note is read by the surgeon, the theatre coordinator and whoever is on tomorrow, so a clear sentence tonight prevents four phone calls in the morning.* |
| 4 · 2:00–2:40 Consequence | Saved as "not fit — optimise potassium". The case card across the theatre list and the readiness board updates immediately. | *Save, and it is everywhere at once. The surgical team see it, the theatre readiness board sees it, and the case stops pretending it is ready. Nobody has to be telephoned and nobody has to remember. That is the entire point of doing it here instead of writing it in the folder.* |
| 5 · 2:40–3:10 Emergencies | Emergency alert arriving on the phone; the emergency pre-anaesthetic assessment opening, shorter, with the essentials only. | *Emergencies reach you the same way but faster: an alert on your phone, and a shorter assessment built for the corridor. Fill in what you have. An emergency review with three fields completed is worth more than a perfect one that never gets written.* |
| 6 · 3:10–3:30 Close | Evening ward round with a phone; then a calm 08:00 start. Presenter. | *One thing to do tomorrow: open the board at six in the evening, not at eight in the morning.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — 08:10, theatre door. An anaesthetist meets a patient for the first time; the observation chart shows a potassium of 6.9. The case is stood down.
2. **Shot 2** (hold) — Anaesthetist Board on a phone: tomorrow's cases grouped by theatre, each showing patient, procedure, anaesthesia type, and a coloured review status.
3. **Shot 3** (hold) — A case opened. Pre-op review form: airway assessment, ASA grade, comorbidities, current medication, the lab results already entered by the team, and a fitness decision with a free-text note.
4. **Shot 4** (hold) — Saved as "not fit — optimise potassium". The case card across the theatre list and the readiness board updates immediately.
5. **Shot 5** (hold) — Emergency alert arriving on the phone; the emergency pre-anaesthetic assessment opening, shorter, with the essentials only.
6. **Shot 6** (hold, worth animating) — Evening ward round with a phone; then a calm 08:00 start. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
He is meeting this patient for the first time at the theatre door. Her potassium is six point nine. The case is now cancelled, the list has lost two hours, and every one of those facts was available last night.

SHOT 2 — hold
The Anaesthetist Board is your list, not the surgeon's. Tomorrow's cases, grouped by theatre, each showing the operation, the planned anaesthetic and whether it has been reviewed. Open it the evening before. Twenty minutes with this screen is the difference between a list that runs and a morning spent apologising.

SHOT 3 — hold
Tap a case for the pre-operative review. The bloods the surgical team entered are already here — you are not chasing them. Add the airway assessment, the ASA grade, the comorbidities and the drugs she is on. Then the decision: fit, fit with conditions, or not fit. Write the reason. That note is read by the surgeon, the theatre coordinator and whoever is on tomorrow, so a clear sentence tonight prevents four phone calls in the morning.

SHOT 4 — hold
Save, and it is everywhere at once. The surgical team see it, the theatre readiness board sees it, and the case stops pretending it is ready. Nobody has to be telephoned and nobody has to remember. That is the entire point of doing it here instead of writing it in the folder.

SHOT 5 — hold
Emergencies reach you the same way but faster: an alert on your phone, and a shorter assessment built for the corridor. Fill in what you have. An emergency review with three fields completed is worth more than a perfect one that never gets written.

SHOT 6 — hold
One thing to do tomorrow: open the board at six in the evening, not at eight in the morning.
```

```
VIDEO 10 - THE ANAESTHETIST BOARD (3 min 30 s)
SCENES: 1 (0:00-0:20) 08:10 at the theatre door, chart showing potassium 6.9, case stood down. 2
```

(0:20-1:05) Anaesthetist Board on a phone; tomorrow grouped by theatre with review status colours. 3 (1:05-2:00) Pre-op review form: airway, ASA, comorbidities, medication, pre-filled labs, fitness decision with note. 4 (2:00-2:40) Saved as "not fit - optimise potassium"; theatre list and readiness board updating. 5 (2:40-3:10) Emergency alert; shorter emergency pre-anaesthetic assessment. 6 (3:10-3:30) Evening ward round with a phone; calm 08:00 start. NARRATION: paste the narration column, in order.

## V11 · Setup and intra-operative monitoring

Anaesthetist | Anaesthetic technician | 3 min 30 s

### THEY WILL BE ABLE TO

- Complete the anaesthesia setup check before the patient arrives.
- Start monitoring and record observations through the case.
- Record drugs given so pharmacy reconciliation is automatic.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18 Hook | Empty theatre before a list. A technician checking a machine; the suction is not connected. | *Seven forty in the morning. Nobody is in the room yet, and the suction is not connected. This is the last five minutes in which that is a small problem.* |
| 2 · 0:18–1:05 Setup | Anesthesia Setup screen: machine check, circuit, suction, airway trolley, drugs drawn, monitoring leads. Each ticked in turn; the theatre readiness indicator turns green. | *Anesthesia Setup is the pre-list check, and it is a list rather than a memory. Machine, circuit, suction, airway trolley, drugs drawn up, monitoring. Tick each as you do it — not before. When the last one is ticked, this theatre shows as anaesthetically ready on the readiness board, and the coordinator stops asking.* |
| 3 · 1:05–2:05 Monitoring | Initialise monitoring for a case. A timeline appears. Observations entered: blood pressure, pulse, saturation, temperature. Each appears as a point on the chart. | *When the case starts, initialise monitoring. You get a timeline for this patient. Enter observations as you take them — pressure, pulse, saturation, temperature — and they build a chart as the case runs. If the connection drops, keep recording. It is held on the machine and sent when the signal returns; nothing you enter is lost.* |
| 4 · 2:05–2:50 Drugs | Drug administration entries added to the same timeline: agent, dose, time. Then a pharmacy view showing the same entries reconciled. | *Record the drugs on the same timeline: what, how much, when. Two things follow. The anaesthetic record writes itself, so there is nothing to reconstruct afterwards. And the pharmacy sees what was actually used against what was issued, which is how the controlled-drug register stops being an argument at the end of the month.* |
| 5 · 2:50–3:20 Handover | End of case. The record closes and appears in the recovery nurse's handover screen. | *At the end, close the record. It travels straight to recovery, so the nurse receiving your patient already has the anaesthetic, the drugs, the losses and your instructions before the trolley arrives.* |
| 6 · 3:20–3:30 Close | Presenter. | *One thing to do tomorrow: tick the setup list as you do it, not from memory at the end.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — Empty theatre before a list. A technician checking a machine; the suction is not connected.
2. **Shot 2** (hold) — Anesthesia Setup screen: machine check, circuit, suction, airway trolley, drugs drawn, monitoring leads. Each ticked in turn; the theatre readiness indicator turns green.
3. **Shot 3** (pan right) — Initialise monitoring for a case. A timeline appears. Observations entered: blood pressure, pulse, saturation, temperature. Each appears as a point on the chart.
4. **Shot 4** (pan right) — Drug administration entries added to the same timeline: agent, dose, time. Then a pharmacy view showing the same entries reconciled.
5. **Shot 5** (hold) — End of case. The record closes and appears in the recovery nurse's handover screen.
6. **Shot 6** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Seven forty in the morning. Nobody is in the room yet, and the suction is not connected. This is the last five minutes in which that is a small problem.

SHOT 2 — hold
Anesthesia Setup is the pre-list check, and it is a list rather than a memory. Machine, circuit, suction, airway trolley, drugs drawn up, monitoring. Tick each as you do it — not before. When the last one is ticked, this theatre shows as anaesthetically ready on the readiness board, and the coordinator stops asking.

SHOT 3 — pan right
When the case starts, initialise monitoring. You get a timeline for this patient. Enter observations as you take them — pressure, pulse, saturation, temperature — and they build a chart as the case runs. If the connection drops, keep recording. It is held on the machine and sent when the signal returns; nothing you enter is lost.

SHOT 4 — pan right
Record the drugs on the same timeline: what, how much, when. Two things follow. The anaesthetic record writes itself, so there is nothing to reconstruct afterwards. And the pharmacy sees what was actually used against what was issued, which is how the controlled-drug register stops being an argument at the end of the month.

SHOT 5 — hold
At the end, close the record. It travels straight to recovery, so the nurse receiving your patient already has the anaesthetic, the drugs, the losses and your instructions before the trolley arrives.

SHOT 6 — pull back
One thing to do tomorrow: tick the setup list as you do it, not from memory at the end.
```

```
VIDEO 11 - SETUP AND INTRA-OPERATIVE MONITORING (3 min 30 s)
SCENES: 1 (0:00-0:18) Empty theatre, technician finds suction not connected. 2 (0:18-1:05) Anesthesia Setup checklist ticked item by item; readiness indicator turning green. 3 (1:05-2:05) Monitoring initialised; observations entered building a live chart. 4 (2:05-2:50) Drug entries on the same timeline; pharmacy reconciliation view. 5 (2:50-3:20) Record closed and appearing in
```

```
recovery handover. 6 (3:20-3:30) Presenter close.
NARRATION: paste the narration column, in order.
```

## V12 · Coverage and the roster

Consultant anaesthetist | Anaesthesia rota-holder | 3 min

**THE RULE THIS VIDEO EXISTS TO TEACH**

An elective case is covered by the anaesthetist rostered to *its own specialty*, on the elective roster, or by nobody. The on-call team never silently covers a planned list. If nobody is rostered, the case reads **"Anaesthetist: not yet assigned"** — and that is a gap somebody must fill, not an error.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:25 Hook | A surgery list where every single case, across eight theatres, names the same anaesthetist. | *Fifteen cases. Eight theatres. One anaesthetist's name on every one of them. He is not doing fifteen lists — he is the on-call consultant, and the elective roster for that day had not been published. The list looked staffed. It was not.* |
| 2 · 0:25–1:10 How cover is decided | Animation: a roster row with a specialty against a name, matching to a case with the same specialty. Then a case with no matching row, showing "not yet assigned" in amber. | *Here is how the app decides. On the elective roster each anaesthetist is recorded against a surgical specialty for the day. A case takes the person rostered to its own specialty. If nobody is rostered to it, the case says, plainly, not yet assigned. It will not quietly borrow the on-call consultant, and it will not borrow a colleague rostered to a different specialty either — that is the same mistake wearing a more convincing name.* |
| 3 · 1:10–1:55 Coverage screen | Anaesthetist Coverage: specialties down the left, tomorrow's rostered names against them, uncovered specialties highlighted amber with their case counts. | *Anaesthetist Coverage is the screen that shows you this before it hurts. Every specialty operating tomorrow, who is covering it, and in amber, the ones with cases and nobody assigned. Open it the afternoon before. An amber row is roughly two minutes of work now, or a stood-down eye list at nine in the morning.* |
| 4 · 1:55–2:35 Assigning | An amber specialty tapped; a list of available anaesthetists; one selected; the row turns green and the cases update. | *Tap an amber row and assign somebody. The cases update immediately. And when the roster itself is published, cases already booked take their proper anaesthetist automatically — you do not go back through them one at a time.* |
| 5 · 2:35–3:00 Close | A coverage screen with every row green. Presenter. | *On call means covering the unplanned. It has never meant covering everybody's elective list. One thing to do tomorrow: publish the elective roster before the cases are booked, not after.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A surgery list where every single case, across eight theatres, names the same anaesthetist.
2. **Shot 2** (pan right) — Animation: a roster row with a specialty against a name, matching to a case with the same specialty. Then a case with no matching row, showing "not yet assigned" in amber.
3. **Shot 3** (push in) — Anaesthetist Coverage: specialties down the left, tomorrow's rostered names against them, uncovered specialties highlighted amber with their case counts.
4. **Shot 4** (hold) — An amber specialty tapped; a list of available anaesthetists; one selected; the row turns green and the cases update.
5. **Shot 5** (hold) — A coverage screen with every row green. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Fifteen cases. Eight theatres. One anaesthetist's name on every one of them. He is not doing
fifteen lists — he is the on-call consultant, and the elective roster for that day had not
been published. The list looked staffed. It was not.

SHOT 2 — pan right
Here is how the app decides. On the elective roster each anaesthetist is recorded against a
surgical specialty for the day. A case takes the person rostered to its own specialty. If
nobody is rostered to it, the case says, plainly, not yet assigned. It will not quietly
borrow the on-call consultant, and it will not borrow a colleague rostered to a different
specialty either — that is the same mistake wearing a more convincing name.

SHOT 3 — push in
Anaesthetist Coverage is the screen that shows you this before it hurts. Every specialty
operating tomorrow, who is covering it, and in amber, the ones with cases and nobody
assigned. Open it the afternoon before. An amber row is roughly two minutes of work now, or a
stood-down eye list at nine in the morning.

SHOT 4 — hold
Tap an amber row and assign somebody. The cases update immediately. And when the roster
itself is published, cases already booked take their proper anaesthetist automatically — you
do not go back through them one at a time.

SHOT 5 — hold
On call means covering the unplanned. It has never meant covering everybody's elective list.
One thing to do tomorrow: publish the elective roster before the cases are booked, not after.
```

```
VIDEO 12 - COVERAGE AND THE ROSTER (3 min)
SCENES: 1 (0:00-0:25) A surgery list where all fifteen cases across eight theatres name one
anaesthetist. 2 (0:25-1:10) Animation of a roster row matching a case by specialty; then a case
with no match showing amber "not yet assigned". 3 (1:10-1:55) Anaesthetist Coverage screen;
uncovered specialties amber with case counts. 4 (1:55-2:35) Amber row tapped, anaesthetist
selected, row turns green, cases update. 5 (2:35-3:00) All-green coverage screen; presenter
close.
NARRATION: paste the narration column, in order.
```

## V13 · The anaesthetic technician

Anaesthetic technician | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A technician carrying a cylinder down a corridor, being stopped twice to answer questions. | *He has been stopped twice before he reached the door. Which theatre is he on today? Is Theatre Three's machine back? Both answers are on his phone.* |
| 2 · 0:18–1:00<br>Your theatre | Technician Coverage screen: theatres listed, each with the technician assigned for the day; the viewer's own theatre highlighted. | *Technician Coverage shows which theatre you are on, and who is on the others. You are rostered to a theatre rather than a specialty, because your work follows the room. If your name is not there, the roster has not been published — say so early, not at half past seven.* |
| 3 · 1:00–1:45<br>Machine and equipment | Anesthesia Setup for the assigned theatre; then Equipment Checkout showing a ventilator booked out and returned. | *Two screens carry your day. Anesthesia Setup is the machine and airway check for your room. Equipment Checkout is anything that moves between theatres — book it out when you take it and back in when it returns. The point is not bureaucracy. It is that when a machine is needed urgently, somebody can see which room it is in instead of opening six doors.* |
| 4 · 1:45–2:20<br>Faults | Fault Alerts: raising a fault on a vaporiser, with a photograph attached. Biomedical engineering receives it. | *If something is broken, raise it in Fault Alerts and photograph it. It goes straight to biomedical engineering with the theatre, the machine and the time. A fault reported this way gets fixed. A fault mentioned in a corridor gets forgotten by the afternoon.* |
| 5 · 2:20–2:30<br>Close | Presenter. | *One thing to do tomorrow: check Technician Coverage before you leave the changing room.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A technician carrying a cylinder down a corridor, being stopped twice to answer questions.
2. **Shot 2** (hold) — Technician Coverage screen: theatres listed, each with the technician assigned for the day; the viewer's own theatre highlighted.
3. **Shot 3** (push in) — Anesthesia Setup for the assigned theatre; then Equipment Checkout showing a ventilator booked out and returned.
4. **Shot 4** (push in, worth animating) — Fault Alerts: raising a fault on a vaporiser, with a photograph attached. Biomedical engineering receives it.
5. **Shot 5** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
He has been stopped twice before he reached the door. Which theatre is he on today? Is
Theatre Three's machine back? Both answers are on his phone.

SHOT 2 — hold
Technician Coverage shows which theatre you are on, and who is on the others. You are
rostered to a theatre rather than a specialty, because your work follows the room. If your
name is not there, the roster has not been published — say so early, not at half past seven.

SHOT 3 — push in
Two screens carry your day. Anesthesia Setup is the machine and airway check for your room.
Equipment Checkout is anything that moves between theatres — book it out when you take it and
back in when it returns. The point is not bureaucracy. It is that when a machine is needed
urgently, somebody can see which room it is in instead of opening six doors.

SHOT 4 — push in
If something is broken, raise it in Fault Alerts and photograph it. It goes straight to
biomedical engineering with the theatre, the machine and the time. A fault reported this way
gets fixed. A fault mentioned in a corridor gets forgotten by the afternoon.

SHOT 5 — pull back
One thing to do tomorrow: check Technician Coverage before you leave the changing room.
```

```
VIDEO 13 - THE ANAESTHETIC TECHNICIAN (2 min 30 s)
SCENES: 1 (0:00-0:18) Technician carrying a cylinder, stopped twice with questions. 2 (0:18-1:00)
Technician Coverage screen with theatres and assigned technicians, own theatre highlighted. 3
(1:00-1:45) Anesthesia Setup for that theatre; Equipment Checkout with a ventilator booked out
and returned. 4 (1:45-2:20) Fault Alerts: raising a vaporiser fault with a photograph. 5 (2:20-
2:30) Presenter close.
NARRATION: paste the narration column, in order.
```

## Part 4 — Nursing

Scrub nurses, circulating nurses, reception, holding area and recovery. Nursing staff touch more screens in this application than any other group, and they are the ones who notice first when something is wrong.

## V14 · Theatre readiness

Scrub nurse | Theatre coordinator | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A coordinator with a clipboard walking into four theatres in turn, asking the same question in each. | *Four rooms, the same question in each, twenty minutes gone, and by the time she reaches the fourth the answer from the first has already changed. There is one screen that answers it for every room at once.* |
| 2 · 0:20–1:10<br>The board | Theatre Readiness: every theatre as a row, with columns for anaesthesia, equipment, packs, sterile supply, cleaning and staff, each a coloured state. | *Theatre Readiness is every room in the suite, side by side. Anaesthesia, equipment, packs, sterile supply, cleaning, staff. Green means somebody has confirmed it. Amber means it is waiting on someone. Red means it is a problem now. Nobody fills this in as a form — it builds itself from what each department has already recorded, which is why the pack list and the CSSD screens matter so much.* |
| 3 · 1:10–2:00<br>Case pack readiness | Case Pack Readiness: tomorrow's cases with their consumable and drug packs, showing which are made up, which are outstanding. | *Case Pack Readiness is the same idea for the lists rather than the rooms. Every case tomorrow, and whether its pack exists yet. Look at this the evening before. A case with no pack at six in the evening is solvable. The same case at half past eight in the morning is a cancellation.* |
| 4 · 2:00–2:45<br>Team check-in | Team Check-in on a phone. Staff tapping in for their theatre; the readiness board's staff column turning green. | *Team Check-in is how the board knows people are actually in the room. Tap in when you arrive at your theatre. It takes two seconds and it replaces the coordinator's walk. It is also the fairest record you have of who was where, on a day somebody claims a theatre was unstaffed.* |
| 5 · 2:45–3:20<br>Milestones | Record Milestones screen; large buttons: patient sent for, patient arrived, anaesthesia started, knife to skin, closing, patient to recovery. One tapped, with the time recorded automatically. | *Through the day, Record Milestones. Big buttons, one tap, the time recorded for you. Sent for, arrived, anaesthesia started, knife to skin, out to recovery. These are what every report and every delay conversation is built from later. Tap them as they happen. Filled in from memory at four in the afternoon, they are fiction.* |
| 6 · 3:20–3:30<br>Close | Board all green; presenter. | *One thing to do tomorrow: open Case Pack Readiness at six in the evening.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A coordinator with a clipboard walking into four theatres in turn, asking the same question in each.
2. **Shot 2** (pan right, worth animating) — Theatre Readiness: every theatre as a row, with columns for anaesthesia, equipment, packs, sterile supply, cleaning and staff, each a coloured state.
3. **Shot 3** (push in) — Case Pack Readiness: tomorrow's cases with their consumable and drug packs, showing which are made up, which are outstanding.
4. **Shot 4** (hold) — Team Check-in on a phone. Staff tapping in for their theatre; the readiness board's staff column turning green.
5. **Shot 5** (hold) — Record Milestones screen; large buttons: patient sent for, patient arrived, anaesthesia started, knife to skin, closing, patient to recovery. One tapped, with the time recorded automatically.
6. **Shot 6** (hold) — Board all green; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Four rooms, the same question in each, twenty minutes gone, and by the time she reaches the fourth the answer from the first has already changed. There is one screen that answers it for every room at once.

SHOT 2 — pan right
Theatre Readiness is every room in the suite, side by side. Anaesthesia, equipment, packs, sterile supply, cleaning, staff. Green means somebody has confirmed it. Amber means it is waiting on someone. Red means it is a problem now. Nobody fills this in as a form — it builds itself from what each department has already recorded, which is why the pack list and the CSSD screens matter so much.

SHOT 3 — push in
Case Pack Readiness is the same idea for the lists rather than the rooms. Every case tomorrow, and whether its pack exists yet. Look at this the evening before. A case with no pack at six in the evening is solvable. The same case at half past eight in the morning is a cancellation.

SHOT 4 — hold
Team Check-in is how the board knows people are actually in the room. Tap in when you arrive at your theatre. It takes two seconds and it replaces the coordinator's walk. It is also the fairest record you have of who was where, on a day somebody claims a theatre was unstaffed.

SHOT 5 — hold
Through the day, Record Milestones. Big buttons, one tap, the time recorded for you. Sent for, arrived, anaesthesia started, knife to skin, out to recovery. These are what every report and every delay conversation is built from later. Tap them as they happen. Filled in from memory at four in the afternoon, they are fiction.

SHOT 6 — hold
One thing to do tomorrow: open Case Pack Readiness at six in the evening.
```

```
VIDEO 14 - THEATRE READINESS (3 min 30 s)
SCENES: 1 (0:00-0:20) Coordinator with clipboard asking the same question in four theatres. 2 (0:20-1:10) Theatre Readiness board: theatres as rows, columns for anaesthesia, equipment, packs,
```

```
sterile supply, cleaning, staff, colour coded. 3 (1:10-2:00) Case Pack Readiness listing
tomorrow's cases and pack status. 4 (2:00-2:45) Team Check-in on a phone; staff column turning
green. 5 (2:45-3:20) Record Milestones with large one-tap buttons and automatic times. 6 (3:20-
3:30) All-green board; presenter.
NARRATION: paste the narration column, in order.
```

## V15 · The WHO checklists

Scrub nurse | Whole theatre team | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A team pausing before incision. Everyone stops. One person reads aloud from a tablet. | *Eleven seconds. That is how long this pause takes, and it is the most evidence-backed eleven seconds in surgery. The app does not replace it. It records that it happened.* |
| 2 · 0:20–1:15<br>Three points | Checklists screen showing the three stages: sign in before anaesthesia, time out before incision, sign out before leaving. Sign-in being completed on a tablet. | *Three points in every case. Sign in, before anaesthesia: identity, site, consent, allergies, airway. Time out, before incision: everyone introduces themselves and the critical steps are stated. Sign out, before the patient leaves: counts, specimens, and what recovery needs to know. Each is a short list on the screen, ticked as it is spoken.* |
| 3 · 1:15–2:00<br>Speak it | Team out loud; the person with the tablet ticking as answers come back, not before. | *One rule decides whether this is worth doing. Tick as the answer is spoken aloud, by the person responsible for it — not in advance, and not by one person reading to themselves. A checklist ticked silently is a checklist that has caught nothing. The value is entirely in the theatre hearing the answer.* |
| 4 · 2:00–2:30<br>Close | Sign-out completing; count confirmed; presenter. | *One thing to do tomorrow: at time out, say your own name and your own role out loud, even if everyone knows you. It gives the newest person in the room permission to speak later, and that is the whole reason it exists.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — A team pausing before incision. Everyone stops. One person reads aloud from a tablet.
2. **Shot 2** (hold) — Checklists screen showing the three stages: sign in before anaesthesia, time out before incision, sign out before leaving. Sign-in being completed on a tablet.
3. **Shot 3** (hold, worth animating) — Team out loud; the person with the tablet ticking as answers come back, not before.
4. **Shot 4** (pull back, worth animating) — Sign-out completing; count confirmed; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 4 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Eleven seconds. That is how long this pause takes, and it is the most evidence-backed eleven seconds in surgery. The app does not replace it. It records that it happened.

SHOT 2 — hold
Three points in every case. Sign in, before anaesthesia: identity, site, consent, allergies, airway. Time out, before incision: everyone introduces themselves and the critical steps are stated. Sign out, before the patient leaves: counts, specimens, and what recovery needs to know. Each is a short list on the screen, ticked as it is spoken.

SHOT 3 — hold
One rule decides whether this is worth doing. Tick as the answer is spoken aloud, by the person responsible for it — not in advance, and not by one person reading to themselves. A checklist ticked silently is a checklist that has caught nothing. The value is entirely in the theatre hearing the answer.

SHOT 4 — pull back
One thing to do tomorrow: at time out, say your own name and your own role out loud, even if everyone knows you. It gives the newest person in the room permission to speak later, and that is the whole reason it exists.
```

```
VIDEO 15 - THE WHO CHECKLISTS (2 min 30 s)
SCENES: 1 (0:00-0:20) Team pausing before incision; one person reading from a tablet. 2 (0:20-1:15) Checklists screen with sign in, time out, sign out; sign-in completed. 3 (1:15-2:00) Team answering aloud while the tablet is ticked in response. 4 (2:00-2:30) Sign-out completing with count confirmed; presenter.
NARRATION: paste the narration column, in order.
```

## V16 · Reception and holding area

Theatre reception | Holding area nurse | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A patient on a trolley in the holding corridor, alone, no notes. A nurse looking through three folders. | *She arrived nine minutes ago. Nobody in this corridor knows which theatre she is for, and her folder is still on the ward. This is the point in the journey where patients get lost.* |
| 2 · 0:20–1:05 Reception | Theatre Reception on a tablet: expected patients today; one marked arrived; identity and folder confirmed against the screen. | *Theatre Reception lists everyone expected today. When the patient reaches the door, find her and mark her arrived. That single tap tells the theatre she is in the building, stops the send-for reminder repeating, and starts the clock that tells you honestly how long she waited.* |
| 3 · 1:05–2:00 Holding assessment | Holding Area assessment: identity, site marked, fasting time, consent sighted, jewellery and prosthetics, allergies, observations. Each confirmed. | *In the holding area, the last check before the doors. Identity, the marked site, when she last ate, consent sighted, jewellery, allergies, and a set of observations. This is the final chance to catch something, and it is deliberately the same list every time so that nothing depends on who is on duty.* |
| 4 · 2:00–2:40 When something is wrong | Fasting time flagged as too short; the case flagged; the anaesthetist's phone alerting. | *If something does not add up, record it as it is. Do not round it, do not assume somebody else has noticed. Flag it here and the anaesthetist knows within seconds, while the patient is still outside the theatre and there are still choices.* |
| 5 · 2:40–3:00 Close | Doors opening, patient going through calmly; presenter. | *One thing to do tomorrow: mark arrival the moment the trolley stops, not when you get a free minute.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A patient on a trolley in the holding corridor, alone, no notes. A nurse looking through three folders.
2. **Shot 2** (hold) — Theatre Reception on a tablet: expected patients today; one marked arrived; identity and folder confirmed against the screen.
3. **Shot 3** (push in) — Holding Area assessment: identity, site marked, fasting time, consent sighted, jewellery and prosthetics, allergies, observations. Each confirmed.
4. **Shot 4** (hold) — Fasting time flagged as too short; the case flagged; the anaesthetist's phone alerting.
5. **Shot 5** (pull back, worth animating) — Doors opening, patient going through calmly; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
She arrived nine minutes ago. Nobody in this corridor knows which theatre she is for, and her folder is still on the ward. This is the point in the journey where patients get lost.

SHOT 2 — hold
Theatre Reception lists everyone expected today. When the patient reaches the door, find her and mark her arrived. That single tap tells the theatre she is in the building, stops the send-for reminder repeating, and starts the clock that tells you honestly how long she waited.

SHOT 3 — push in
In the holding area, the last check before the doors. Identity, the marked site, when she last ate, consent sighted, jewellery, allergies, and a set of observations. This is the final chance to catch something, and it is deliberately the same list every time so that nothing depends on who is on duty.

SHOT 4 — hold
If something does not add up, record it as it is. Do not round it, do not assume somebody else has noticed. Flag it here and the anaesthetist knows within seconds, while the patient is still outside the theatre and there are still choices.

SHOT 5 — pull back
One thing to do tomorrow: mark arrival the moment the trolley stops, not when you get a free minute.
```

```
VIDEO 16 - RECEPTION AND HOLDING AREA (3 min)
SCENES: 1 (0:00-0:20) Patient alone on a trolley in a corridor; nurse searching three folders. 2 (0:20-1:05) Theatre Reception on a tablet; patient marked arrived. 3 (1:05-2:00) Holding Area assessment: identity, marked site, fasting, consent sighted, jewellery, allergies, observations. 4 (2:00-2:40) Short fasting time flagged; anaesthetist's phone alerting. 5 (2:40-3:00) Doors opening calmly; presenter.
NARRATION: paste the narration column, in order.
```

## V17 · Calling for the patient

Theatre nurse | Porter | Ward nurse | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | Theatre ready, team scrubbed, empty table. Cut to a ward where the patient sits fully dressed, nobody having asked for her. | *The theatre has been ready for twenty-five minutes. On the ward she is still in her own clothes, because nobody has asked for her. Neither room knows about the other.* |
| 2 · 0:20–1:10<br>Call for patient | Call for Patient screen: today's cases in order; one tapped; a confirmation. Corridor speaker announcing by name and ward; the porter's phone buzzing. | *Call for Patient. Find the case and tap it. Three things happen at once: the radio announces her by name and ward, the porters are alerted on their phones, and the ward sees the request on their own screen. One tap replaces three phone calls, and the time is recorded, so if she takes forty minutes to arrive, that is visible rather than argued about.* |
| 3 · 1:10–1:50<br>The hour before | An automatic reminder an hour before the scheduled start, announced over the radio and shown on the board. | *You also get help before you ask. About an hour before each case is due, the app announces it by itself, so the ward can start getting her ready. That reminder is why the first case of the afternoon is now on time more often than it used to be.* |
| 4 · 1:50–2:20<br>Arrival | Patient arrives; reception marks arrival; the repeating announcement stops. | *When she arrives, reception marks it, and the announcement stops repeating. If you ever hear the same call over and over, it means nobody has marked the arrival — and that is worth fixing straight away, because an alert people learn to ignore is worse than no alert at all.* |
| 5 · 2:20–2:30<br>Close | Presenter. | *One thing to do tomorrow: call for the next patient while the current one is still being closed.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — Theatre ready, team scrubbed, empty table. Cut to a ward where the patient sits fully dressed, nobody having asked for her.
2. **Shot 2** (hold) — Call for Patient screen: today's cases in order; one tapped; a confirmation. Corridor speaker announcing by name and ward; the porter's phone buzzing.
3. **Shot 3** (hold) — An automatic reminder an hour before the scheduled start, announced over the radio and shown on the board.
4. **Shot 4** (push in, worth animating) — Patient arrives; reception marks arrival; the repeating announcement stops.
5. **Shot 5** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
The theatre has been ready for twenty-five minutes. On the ward she is still in her own clothes, because nobody has asked for her. Neither room knows about the other.

SHOT 2 — hold
Call for Patient. Find the case and tap it. Three things happen at once: the radio announces her by name and ward, the porters are alerted on their phones, and the ward sees the request on their own screen. One tap replaces three phone calls, and the time is recorded, so if she takes forty minutes to arrive, that is visible rather than argued about.

SHOT 3 — hold
You also get help before you ask. About an hour before each case is due, the app announces it by itself, so the ward can start getting her ready. That reminder is why the first case of the afternoon is now on time more often than it used to be.

SHOT 4 — push in
When she arrives, reception marks it, and the announcement stops repeating. If you ever hear the same call over and over, it means nobody has marked the arrival — and that is worth fixing straight away, because an alert people learn to ignore is worse than no alert at all.

SHOT 5 — pull back
One thing to do tomorrow: call for the next patient while the current one is still being closed.
```

```
VIDEO 17 - CALLING FOR THE PATIENT (2 min 30 s)
SCENES: 1 (0:00-0:20) Ready theatre with an empty table; ward with the patient still dressed. 2 (0:20-1:10) Call for Patient tapped; corridor speaker announcing name and ward; porter's phone buzzing; ward screen updating. 3 (1:10-1:50) Automatic reminder an hour before the scheduled start. 4 (1:50-2:20) Arrival marked; repeating announcement stops. 5 (2:20-2:30) Presenter.
NARRATION: paste the narration column, in order.
```

## V18 · Nurse handover

Scrub nurse | Circulating nurse | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | Shift change. Two nurses talking quickly in a doorway while a case is still running. | *Forty seconds in a doorway, with a case still running behind them. Everything she has learned in six hours has to cross that gap. Most of it will not.* |
| 2 · 0:20–1:20<br>The handover | Nurse Handover screen: theatre, cases done, cases remaining, equipment faults, stock used and low, patients of concern, notes for the next shift. | *Nurse Handover gives that conversation a shape. What has been done, what is still to come, what broke, what has run low, and which patient you are worried about. Write it as the shift runs, not at the end — a line at a time, when it happens. It takes ten seconds each and it means the person following you starts where you finished instead of where you started.* |
| 3 · 1:20–2:05<br>Receiving | Incoming nurse opening the handover on a phone in the changing room, before entering the theatre. | *And if you are coming on, read it before you walk in. Two minutes in the changing room, and you enter the room already knowing that the diathermy is intermittent and that the second case has a difficult airway. That is a different shift from the one that begins with a whispered handover in a doorway.* |
| 4 · 2:05–2:30<br>Close | Calm shift change; presenter. | *One thing to do tomorrow: add one line to the handover as soon as anything goes wrong, while you are still annoyed about it.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — Shift change. Two nurses talking quickly in a doorway while a case is still running.
2. **Shot 2** (hold) — Nurse Handover screen: theatre, cases done, cases remaining, equipment faults, stock used and low, patients of concern, notes for the next shift.
3. **Shot 3** (hold, worth animating) — Incoming nurse opening the handover on a phone in the changing room, before entering the theatre.
4. **Shot 4** (pull back, worth animating) — Calm shift change; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 4 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
Forty seconds in a doorway, with a case still running behind them. Everything she has learned in six hours has to cross that gap. Most of it will not.

SHOT 2 — hold
Nurse Handover gives that conversation a shape. What has been done, what is still to come, what broke, what has run low, and which patient you are worried about. Write it as the shift runs, not at the end — a line at a time, when it happens. It takes ten seconds each and it means the person following you starts where you finished instead of where you started.

SHOT 3 — hold
And if you are coming on, read it before you walk in. Two minutes in the changing room, and you enter the room already knowing that the diathermy is intermittent and that the second case has a difficult airway. That is a different shift from the one that begins with a whispered handover in a doorway.

SHOT 4 — pull back
One thing to do tomorrow: add one line to the handover as soon as anything goes wrong, while you are still annoyed about it.
```

```
VIDEO 18 - NURSE HANDOVER (2 min 30 s)
SCENES: 1 (0:00-0:20) Two nurses handing over quickly in a doorway during a running case. 2 (0:20-1:20) Nurse Handover screen: cases done and remaining, faults, stock, patients of concern, notes. 3 (1:20-2:05) Incoming nurse reading it on a phone in the changing room. 4 (2:05-2:30) Calm shift change; presenter.
NARRATION: paste the narration column, in order.
```

## V19 · Recovery (PACU)

Recovery room nurse | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A patient wheeled into recovery. The nurse receiving has no notes, and the anaesthetist has already gone back in. | *She has thirty seconds of handover and no notes, and the anaesthetist is already back in theatre. She is about to look after somebody she knows nothing about.* |
| 2 · 0:20–1:05 Arrival | PACU screen. The patient already listed, with the anaesthetic record, drugs given, losses, and the anaesthetist's instructions attached. | *Open PACU and she is already there. The anaesthetic that was used, the drugs given and when, blood loss, and what the anaesthetist wants watched. It arrived when the record was closed in theatre. You are not waiting for paper and you are not relying on a doorway conversation.* |
| 3 · 1:05–2:00 Observations | Recording observations at intervals; the trend building; pain score and nausea recorded; a medication given and logged. | *Record observations as you take them and they build a trend rather than a page of numbers. Pain and nausea go here too, because they are the two things that decide how the patient remembers the whole operation. Anything you give is logged against her, so the ward sees exactly what she has had and when it is next due.* |
| 4 · 2:00–2:50 Discharge | Discharge criteria scored; the score reaching threshold; destination confirmed; ward notified automatically. | *Discharge is a score, not a feeling. Work through the criteria — consciousness, breathing, circulation, pain, nausea, mobility. When she meets them, record the destination and the ward is told she is coming. If she does not meet them, that is recorded too, and it is the strongest thing you can hold when somebody is pressing you for the bay.* |
| 5 · 2:50–3:20 Escalation | A deteriorating patient; an alert raised from the PACU screen; the anaesthetist's phone alerting. | *If a patient deteriorates, raise it from this screen. It alerts the anaesthetist directly rather than sending somebody to find one, and it timestamps the moment you were worried — which matters afterwards, for you.* |
| 6 · 3:20–3:30 Close | Presenter. | *One thing to do tomorrow: open PACU before the trolley arrives, not after.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A patient wheeled into recovery. The nurse receiving has no notes, and the anaesthetist has already gone back in.
2. **Shot 2** (hold) — PACU screen. The patient already listed, with the anaesthetic record, drugs given, losses, and the anaesthetist's instructions attached.
3. **Shot 3** (push in) — Recording observations at intervals; the trend building; pain score and nausea recorded; a medication given and logged.
4. **Shot 4** (push in) — Discharge criteria scored; the score reaching threshold; destination confirmed; ward notified automatically.
5. **Shot 5** (hold) — A deteriorating patient; an alert raised from the PACU screen; the anaesthetist's phone alerting.
6. **Shot 6** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
She has thirty seconds of handover and no notes, and the anaesthetist is already back in
theatre. She is about to look after somebody she knows nothing about.

SHOT 2 — hold
Open PACU and she is already there. The anaesthetic that was used, the drugs given and when,
blood loss, and what the anaesthetist wants watched. It arrived when the record was closed in
theatre. You are not waiting for paper and you are not relying on a doorway conversation.

SHOT 3 — push in
Record observations as you take them and they build a trend rather than a page of numbers.
Pain and nausea go here too, because they are the two things that decide how the patient
remembers the whole operation. Anything you give is logged against her, so the ward sees
exactly what she has had and when it is next due.

SHOT 4 — push in
Discharge is a score, not a feeling. Work through the criteria — consciousness, breathing,
circulation, pain, nausea, mobility. When she meets them, record the destination and the ward
is told she is coming. If she does not meet them, that is recorded too, and it is the
strongest thing you can hold when somebody is pressing you for the bay.

SHOT 5 — hold
If a patient deteriorates, raise it from this screen. It alerts the anaesthetist directly
rather than sending somebody to find one, and it timestamps the moment you were worried —
which matters afterwards, for you.

SHOT 6 — pull back
One thing to do tomorrow: open PACU before the trolley arrives, not after.
```

```
VIDEO 19 - RECOVERY (PACU) (3 min 30 s)
SCENES: 1 (0:00-0:20) Patient wheeled into recovery, nurse with no notes, anaesthetist leaving. 2
(0:20-1:05) PACU screen already showing the patient with anaesthetic record, drugs, losses,
instructions. 3 (1:05-2:00) Observations recorded building a trend; pain and nausea; medication
logged. 4 (2:00-2:50) Discharge criteria scored; threshold met; destination set; ward notified. 5
```

```
(2:50-3:20) Deterioration; alert raised; anaesthetist's phone alerting. 6 (3:20-3:30) Presenter.
NARRATION: paste the narration column, in order.
```

# Part 5 — Pharmacy, blood bank and laboratory

Three departments that receive work automatically the moment a case is booked, and whose answers decide whether the morning list runs.

## V20 · Pharmacy and drug packs

Pharmacist | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | Pharmacy window, 07:50. A house officer arrives out of breath with a handwritten list for a case starting at eight. | *Ten to eight, a handwritten list, and a case that starts in ten minutes. This used to be every morning. It does not have to be, because that list was written three weeks ago and nobody sent it to you.* |
| 2 · 0:20–1:10<br>Work arrives | Pharmacy screen: tomorrow's cases, each with its drug and dressing list, patient, procedure and start time. A notification badge on the menu item. | *Open Pharmacy and the work is already here. Every case booked for tomorrow, with the drugs and dressings the surgeon asked for, the patient, the operation and the time it starts. It arrived the moment the case was booked — often days in advance. You are notified when a new one lands, so you are not refreshing a screen.* |
| 3 · 1:10–2:00<br>Preparing | A case opened; items ticked as picked; one item marked unavailable with a note; status moves to packed. | *Work down the list, ticking as you pick. If something is not available, say so here rather than leaving a gap — mark it and write what you have instead. That single note appears on the surgeon's case and on the theatre readiness board tonight, which is the difference between a substitution agreed calmly in the evening and an argument at the theatre door.* |
| 4 · 2:00–2:40<br>The patient code | A patient at the window handing over a card with a pharmacy code. The code typed in; the correct pack appears immediately. | *When the patient comes to the window she brings a code the surgeon gave her at booking. Type it in and her pack comes straight up — no folder number, no spelling her name three ways, no phone call to the ward. Encourage patients to bring it. It is the fastest thing at this counter.* |
| 5 · 2:40–3:15<br>Approvals and tracking | Rx Approvals screen with a controlled item awaiting sign-off; then Med Tracking showing issued against administered for a completed case. | *Two more screens. Rx Approvals holds anything needing a pharmacist's sign-off before it goes. Med Tracking shows what was issued against what the anaesthetist recorded as given, case by case. That reconciliation used to be a monthly argument reconstructed from memory. It is now a screen you can open on the day.* |
| 6 · 3:15–3:30<br>Close | Calm pharmacy at 07:50, packs already labelled and waiting. Presenter. | *One thing to do tomorrow: open Pharmacy at four in the afternoon and prepare the morning list before you go home.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — Pharmacy window, 07:50. A house officer arrives out of breath with a handwritten list for a case starting at eight.
2. **Shot 2** (hold) — Pharmacy screen: tomorrow's cases, each with its drug and dressing list, patient, procedure and start time. A notification badge on the menu item.
3. **Shot 3** (push in) — A case opened; items ticked as picked; one item marked unavailable with a note; status moves to packed.
4. **Shot 4** (hold, worth animating) — A patient at the window handing over a card with a pharmacy code. The code typed in; the correct pack appears immediately.
5. **Shot 5** (hold) — Rx Approvals screen with a controlled item awaiting sign-off; then Med Tracking showing issued against administered for a completed case.
6. **Shot 6** (pull back, worth animating) — Calm pharmacy at 07:50, packs already labelled and waiting. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Ten to eight, a handwritten list, and a case that starts in ten minutes. This used to be every morning. It does not have to be, because that list was written three weeks ago and nobody sent it to you.

SHOT 2 — hold
Open Pharmacy and the work is already here. Every case booked for tomorrow, with the drugs and dressings the surgeon asked for, the patient, the operation and the time it starts. It arrived the moment the case was booked — often days in advance. You are notified when a new one lands, so you are not refreshing a screen.

SHOT 3 — push in
Work down the list, ticking as you pick. If something is not available, say so here rather than leaving a gap — mark it and write what you have instead. That single note appears on the surgeon's case and on the theatre readiness board tonight, which is the difference between a substitution agreed calmly in the evening and an argument at the theatre door.

SHOT 4 — hold
When the patient comes to the window she brings a code the surgeon gave her at booking. Type it in and her pack comes straight up — no folder number, no spelling her name three ways, no phone call to the ward. Encourage patients to bring it. It is the fastest thing at this counter.

SHOT 5 — hold
Two more screens. Rx Approvals holds anything needing a pharmacist's sign-off before it goes. Med Tracking shows what was issued against what the anaesthetist recorded as given, case by case. That reconciliation used to be a monthly argument reconstructed from memory. It is now a screen you can open on the day.

SHOT 6 — pull back
One thing to do tomorrow: open Pharmacy at four in the afternoon and prepare the morning list before you go home.
```

VIDEO 20 - PHARMACY AND DRUG PACKS (3 min 30 s)
SCENES: 1 (0:00-0:20) Pharmacy window at 07:50; breathless house officer with a handwritten list. 2 (0:20-1:10) Pharmacy screen: tomorrow's cases with drug lists, patient, procedure, start time; notification badge. 3 (1:10-2:00) A case opened; items ticked; one marked unavailable with a substitution note; status set to packed. 4 (2:00-2:40) Patient hands over a card with a pharmacy code; typed in; the pack appears. 5 (2:40-3:15) Rx Approvals with a controlled item; Med Tracking showing issued against administered. 6 (3:15-3:30) Calm pharmacy, packs labelled and waiting.
NARRATION: paste the narration column, in order.

## V21 · Blood bank

Blood bank staff | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A theatre mid-case, surgeon asking for blood. A runner sent down a corridor. Clock ticking. | *The blood was requested at the moment it was needed. Cross-matching takes time that this patient does not have. The request should have reached you yesterday, and the system already knew.* |
| 2 · 0:20–1:05 Requests | Blood Bank screen: requests listed with patient, folder number, group, units, the surgery date and whether it is an emergency. | *Every request carries the patient, the folder number, the group, how many units, and when the operation is. Emergency requests sit at the top and look different, because they are. Acknowledge a request as you pick it up — the theatre can then see that a human being has it, which stops the runner being sent.* |
| 3 · 1:05–1:50 Cross-match | A request opened; cross-match recorded as completed; units reserved against the patient; status visible on the theatre board. | *Record the cross-match when it is done and reserve the units against that patient. The theatre readiness board updates, so the surgical team can see blood is ready without telephoning. If a group is unavailable or a unit is short, record that instead. An honest shortage seen at four in the afternoon can be solved. The same shortage discovered at incision cannot.* |
| 4 · 1:50–2:35 Emergencies | An emergency request arriving with an alert; the same screen, fewer steps; units issued urgently and logged. | *Emergency requests alert you directly rather than waiting to be noticed. The process is the same, with fewer steps. Everything you issue is logged against the patient and the case, so the return, the traceability and the audit are done as a by-product of the work rather than as a second job at the end of the week.* |
| 5 · 2:35–3:00 Close | Blood ready in a cool box before the list starts. Presenter. | *One thing to do tomorrow: acknowledge every request the moment you read it, even before you start work on it.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A theatre mid-case, surgeon asking for blood. A runner sent down a corridor. Clock ticking.
2. **Shot 2** (hold) — Blood Bank screen: requests listed with patient, folder number, group, units, the surgery date and whether it is an emergency.
3. **Shot 3** (hold) — A request opened; cross-match recorded as completed; units reserved against the patient; status visible on the theatre board.
4. **Shot 4** (hold) — An emergency request arriving with an alert; the same screen, fewer steps; units issued urgently and logged.
5. **Shot 5** (hold, worth animating) — Blood ready in a cool box before the list starts. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
The blood was requested at the moment it was needed. Cross-matching takes time that this patient does not have. The request should have reached you yesterday, and the system already knew.

SHOT 2 — hold
Every request carries the patient, the folder number, the group, how many units, and when the operation is. Emergency requests sit at the top and look different, because they are. Acknowledge a request as you pick it up — the theatre can then see that a human being has it, which stops the runner being sent.

SHOT 3 — hold
Record the cross-match when it is done and reserve the units against that patient. The theatre readiness board updates, so the surgical team can see blood is ready without telephoning. If a group is unavailable or a unit is short, record that instead. An honest shortage seen at four in the afternoon can be solved. The same shortage discovered at incision cannot.

SHOT 4 — hold
Emergency requests alert you directly rather than waiting to be noticed. The process is the same, with fewer steps. Everything you issue is logged against the patient and the case, so the return, the traceability and the audit are done as a by-product of the work rather than as a second job at the end of the week.

SHOT 5 — hold
One thing to do tomorrow: acknowledge every request the moment you read it, even before you start work on it.
```

```
VIDEO 21 - BLOOD BANK (3 min)
SCENES: 1 (0:00-0:20) Mid-case request for blood; runner sent down a corridor; clock ticking. 2 (0:20-1:05) Blood Bank request list with patient, folder, group, units, surgery date, emergencies at top. 3 (1:05-1:50) Cross-match recorded; units reserved; theatre board updating. 4 (1:50-2:35) Emergency request arriving with an alert; urgent issue logged. 5 (2:35-3:00) Blood ready in a cool box before the list starts.
NARRATION: paste the narration column, in order.
```

## V22 · Laboratory and emergency workup

Laboratory staff | Emergency lab scientist | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A result printed and sitting in a tray. Cut to a theatre where the same result is being asked for by phone. | *This result has existed for two hours. It is in a tray, twelve metres from a theatre that is currently telephoning to ask for it. The only thing missing is somebody typing it once.* |
| 2 · 0:20–1:05<br>What is outstanding | Emergency Lab Workup screen: patients with outstanding investigations, showing which tests and how urgent, ordered by surgery time. | *Emergency Lab Workup shows the patients waiting on you, which tests, and when their operation is. It is ordered by how soon they are needed, not by when the sample arrived, which is a different order and usually a better one.* |
| 3 · 1:05–2:00<br>Entering results | A patient opened; haemoglobin, potassium, sodium, creatinine entered with sample time; viral screen results set. Each one clears an outstanding item on the case. | *Enter the result and the time the sample was taken. Watch what happens to the case: each value you enter removes an outstanding item from the surgeon's list, the anaesthetist's board and the readiness screen at the same moment. One entry, three departments told. That is the whole return on the thirty seconds it takes.* |
| 4 · 2:00–2:40<br>Critical values | A markedly abnormal potassium entered; the case flags; the anaesthetist and surgical team alerted. | *If a value is dangerous, entering it is the fastest way to raise the alarm. The case flags immediately and the anaesthetist is told, without you finding a phone number or hoping somebody checks the tray. You have done the most useful thing you can do with that number in the moment you know it.* |
| 5 · 2:40–3:00<br>Close | Empty tray; presenter. | *One thing to do tomorrow: enter results as they are validated, rather than in a batch at the end of the shift.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A result printed and sitting in a tray. Cut to a theatre where the same result is being asked for by phone.
2. **Shot 2** (hold) — Emergency Lab Workup screen: patients with outstanding investigations, showing which tests and how urgent, ordered by surgery time.
3. **Shot 3** (hold) — A patient opened; haemoglobin, potassium, sodium, creatinine entered with sample time; viral screen results set. Each one clears an outstanding item on the case.
4. **Shot 4** (push in, worth animating) — A markedly abnormal potassium entered; the case flags; the anaesthetist and surgical team alerted.
5. **Shot 5** (pull back, worth animating) — Empty tray; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
This result has existed for two hours. It is in a tray, twelve metres from a theatre that is currently telephoning to ask for it. The only thing missing is somebody typing it once.

SHOT 2 — hold
Emergency Lab Workup shows the patients waiting on you, which tests, and when their operation is. It is ordered by how soon they are needed, not by when the sample arrived, which is a different order and usually a better one.

SHOT 3 — hold
Enter the result and the time the sample was taken. Watch what happens to the case: each value you enter removes an outstanding item from the surgeon's list, the anaesthetist's board and the readiness screen at the same moment. One entry, three departments told. That is the whole return on the thirty seconds it takes.

SHOT 4 — push in
If a value is dangerous, entering it is the fastest way to raise the alarm. The case flags immediately and the anaesthetist is told, without you finding a phone number or hoping somebody checks the tray. You have done the most useful thing you can do with that number in the moment you know it.

SHOT 5 — pull back
One thing to do tomorrow: enter results as they are validated, rather than in a batch at the end of the shift.
```

```
VIDEO 22 - LABORATORY AND EMERGENCY WORKUP (3 min)
SCENES: 1 (0:00-0:20) A printed result in a tray; a theatre telephoning for the same result. 2 (0:20-1:05) Emergency Lab Workup listing patients with outstanding tests ordered by surgery time. 3 (1:05-2:00) Results entered with sample times; outstanding items clearing across three screens. 4 (2:00-2:40) A dangerous potassium entered; case flags; anaesthetist alerted. 5 (2:40-3:00) Empty tray; presenter.
NARRATION: paste the narration column, in order.
```

## Part 6 — Sterile services, supply and logistics

The departments that make the list physically possible. None of them are clinical, and every one of them can stop a case.

## V23 · CSSD

CSSD staff | CSSD supervisor | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A theatre waiting; a set opened and found to be the wrong one. The scrub nurse's face. | *The set is open. It is the wrong one, and now it is also unusable. Somewhere in the department the right one is sitting sterile on a shelf, and the only thing that failed was knowing.* |
| 2 · 0:20–1:10 Inventory | CSSD Inventory: instrument sets with location, sterile status and expiry. | *CSSD Inventory is every set, where it is, whether it is sterile and when that expires. Keep it accurate as things move and it answers the question the theatre keeps ringing to ask. A set whose record says sterile but is actually in the washer is worse than no record at all, so record the move as you make it.* |
| 3 · 1:10–2:00 Sterilisation cycles | Sterilization screen: a load created, contents listed, cycle parameters recorded, indicator result entered, load released. | *Every cycle is recorded: what went in, the parameters, the indicator result, and who released the load. That is your traceability. If a question is ever asked about an instrument used in a particular case — and one day it will be — this is the record that answers it in a minute instead of a week.* |
| 4 · 2:00–2:40 Readiness | CSSD Readiness: tomorrow's cases against the sets they need, showing which are ready and which are still processing. | *CSSD Readiness turns tomorrow's operating list into your work list. The sets each case needs, and whether they will be ready. Look at it in the afternoon. A set that will not be ready by nine tomorrow is a phone call you can make today, to a surgeon who still has options.* |
| 5 · 2:40–3:00 Close | Correct set delivered and opened. Presenter. | *One thing to do tomorrow: open CSSD Readiness at two in the afternoon and work backwards from the morning list.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — A theatre waiting; a set opened and found to be the wrong one. The scrub nurse's face.
2. **Shot 2** (push in) — CSSD Inventory: instrument sets with location, sterile status and expiry.
3. **Shot 3** (hold) — Sterilization screen: a load created, contents listed, cycle parameters recorded, indicator result entered, load released.
4. **Shot 4** (push in) — CSSD Readiness: tomorrow's cases against the sets they need, showing which are ready and which are still processing.
5. **Shot 5** (pull back, worth animating) — Correct set delivered and opened. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
The set is open. It is the wrong one, and now it is also unusable. Somewhere in the
department the right one is sitting sterile on a shelf, and the only thing that failed was
knowing.

SHOT 2 — push in
CSSD Inventory is every set, where it is, whether it is sterile and when that expires. Keep
it accurate as things move and it answers the question the theatre keeps ringing to ask. A
set whose record says sterile but is actually in the washer is worse than no record at all,
so record the move as you make it.

SHOT 3 — hold
Every cycle is recorded: what went in, the parameters, the indicator result, and who released
the load. That is your traceability. If a question is ever asked about an instrument used in
a particular case — and one day it will be — this is the record that answers it in a minute
instead of a week.

SHOT 4 — push in
CSSD Readiness turns tomorrow's operating list into your work list. The sets each case needs,
and whether they will be ready. Look at it in the afternoon. A set that will not be ready by
nine tomorrow is a phone call you can make today, to a surgeon who still has options.

SHOT 5 — pull back
One thing to do tomorrow: open CSSD Readiness at two in the afternoon and work backwards from
the morning list.
```

```
VIDEO 23 - CSSD (3 min)
SCENES: 1 (0:00-0:20) Wrong set opened in theatre; scrub nurse's reaction. 2 (0:20-1:10) CSSD
Inventory: sets with location, sterile status, expiry. 3 (1:10-2:00) Sterilization: load created,
contents, parameters, indicator result, release. 4 (2:00-2:40) CSSD Readiness matching tomorrow's
cases to required sets. 5 (2:40-3:00) Correct set delivered and opened.
NARRATION: paste the narration column, in order.
```

## V24 · Theatre store and supply

Theatre store keeper | Theatre supply unit | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A store with full shelves. A theatre cancelled for a missing item that is on a shelf in the next room, in a box nobody opened. | *That item is in this room. It has been for three weeks. The case was cancelled anyway, because the shelf knew and the system did not.* |
| 2 · 0:20–1:05 Inventory and movements | Inventory screen: items, quantities, reorder levels, low-stock highlighted. A stock movement being recorded as items are issued. | *Inventory holds what you have, what the reorder level is, and highlights what is running low. The balance is not typed — it is calculated from movements. So record issues and receipts as they happen, and the number stays true by itself. Overwrite a balance by hand and you have hidden the very thing the screen exists to show.* |
| 3 · 1:05–2:00 Requests from theatre | Theatre Supply Unit: consumable requests from booked cases, grouped by case and date, with the standard theatre pack marked separately from surgeon additions. | *Requests arrive from bookings without anybody raising them. Each case shows the standard theatre pack and, marked separately, what the surgeon added for this patient. That distinction matters: the additions are the ones that are unusual, the ones you may not stock, and the ones worth reading first.* |
| 4 · 2:00–2:45 Sub-stores and reports | Sub-Stores view showing satellite stock; then Supply Reports with consumption trends; then Bulk Import loading a supplier delivery from a spreadsheet. | *Sub-Stores tracks what has gone out to satellite stores so a theatre cupboard is not invisible stock. Supply Reports shows consumption over time, which is the argument you need when asking for a budget. And Bulk Import loads a delivery from a spreadsheet rather than one line at a time.* |
| 5 · 2:45–3:20 Codes | A patient at the store window presenting a consumable pack code; the pack list appearing. | *Patients arrive with a consumable pack code from their surgeon. Type it in and their list appears. It saves them explaining and saves you searching.* |
| 6 · 3:20–3:30 Close | Presenter. | *One thing to do tomorrow: record every issue as you hand it over, not at the end of the day.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A store with full shelves. A theatre cancelled for a missing item that is on a shelf in the next room, in a box nobody opened.
2. **Shot 2** (hold) — Inventory screen: items, quantities, reorder levels, low-stock highlighted. A stock movement being recorded as items are issued.
3. **Shot 3** (push in, worth animating) — Theatre Supply Unit: consumable requests from booked cases, grouped by case and date, with the standard theatre pack marked separately from surgeon additions.
4. **Shot 4** (push in) — Sub-Stores view showing satellite stock; then Supply Reports with consumption trends; then Bulk Import loading a supplier delivery from a spreadsheet.
5. **Shot 5** (hold, worth animating) — A patient at the store window presenting a consumable pack code; the pack list appearing.
6. **Shot 6** (pull back, worth animating) — Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
That item is in this room. It has been for three weeks. The case was cancelled anyway,
because the shelf knew and the system did not.

SHOT 2 — hold
Inventory holds what you have, what the reorder level is, and highlights what is running low.
The balance is not typed — it is calculated from movements. So record issues and receipts as
they happen, and the number stays true by itself. Overwrite a balance by hand and you have
hidden the very thing the screen exists to show.

SHOT 3 — push in
Requests arrive from bookings without anybody raising them. Each case shows the standard
theatre pack and, marked separately, what the surgeon added for this patient. That
distinction matters: the additions are the ones that are unusual, the ones you may not stock,
and the ones worth reading first.

SHOT 4 — push in
Sub-Stores tracks what has gone out to satellite stores so a theatre cupboard is not
invisible stock. Supply Reports shows consumption over time, which is the argument you need
when asking for a budget. And Bulk Import loads a delivery from a spreadsheet rather than one
line at a time.

SHOT 5 — hold
Patients arrive with a consumable pack code from their surgeon. Type it in and their list
appears. It saves them explaining and saves you searching.

SHOT 6 — pull back
One thing to do tomorrow: record every issue as you hand it over, not at the end of the day.
```

```
VIDEO 24 - THEATRE STORE AND SUPPLY (3 min 30 s)
SCENES: 1 (0:00-0:20) Full store shelves; a case cancelled for an item that was in the next room.
2 (0:20-1:05) Inventory with reorder levels and low stock; a movement recorded on issue. 3 (1:05-
2:00) Theatre Supply Unit: requests grouped by case, standard pack separated from surgeon
additions. 4 (2:00-2:45) Sub-Stores; Supply Reports consumption trends; Bulk Import from a
```

```
spreadsheet. 5 (2:45-3:20) Patient presenting a consumable pack code at the window. 6 (3:20-3:30)
Presenter.
NARRATION: paste the narration column, in order.
```

## V25 · Consumable packs

Consumable pack provider | Scrub care provider | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A phone buzzing on a bench in the store at 17:40. The screen reads: new consumables pre-pack request. | *Twenty to six. The store is still open, the shelves are still reachable, and a request has just arrived for a case at nine tomorrow morning. This is the moment the case is either going to run or not.* |
| 2 · 0:18–1:10<br>The work list | Consumable Packs screen: cases for tomorrow, each with its item list, quantities and the standard pack marked. | *Your screen is tomorrow's cases and what each one needs. You are told as soon as a case is booked, which is often days ahead. Work the list the evening before — every pack made up tonight is a case that cannot be delayed for a pack in the morning.* |
| 3 · 1:10–1:55<br>Packing | Items ticked as packed; one item unavailable, marked with a note; the pack marked ready; the case status updating on the readiness board. | *Tick as you pack. If something is missing, mark it and write what you have put in instead — do not leave it blank and do not substitute silently. That note reaches the surgeon tonight. A surgeon told at six in the evening will adapt. A surgeon who discovers it at incision will not, and should not have to.* |
| 4 · 1:55–2:20<br>Codes | A patient presenting a code; the pack found instantly; handed over and recorded. | *When a patient brings a code, type it in and her pack comes up. Record the hand-over so the theatre knows it has left you.* |
| 5 · 2:20–2:30<br>Close | A row of labelled packs ready on a shelf. Presenter. | *One thing to do tomorrow: clear your list before you go home, not before you go to lunch.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A phone buzzing on a bench in the store at 17:40. The screen reads: new consumables pre-pack request.
2. **Shot 2** (hold) — Consumable Packs screen: cases for tomorrow, each with its item list, quantities and the standard pack marked.
3. **Shot 3** (hold) — Items ticked as packed; one item unavailable, marked with a note; the pack marked ready; the case status updating on the readiness board.
4. **Shot 4** (push in, worth animating) — A patient presenting a code; the pack found instantly; handed over and recorded.
5. **Shot 5** (pull back, worth animating) — A row of labelled packs ready on a shelf. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Twenty to six. The store is still open, the shelves are still reachable, and a request has just arrived for a case at nine tomorrow morning. This is the moment the case is either going to run or not.

SHOT 2 — hold
Your screen is tomorrow's cases and what each one needs. You are told as soon as a case is booked, which is often days ahead. Work the list the evening before — every pack made up tonight is a case that cannot be delayed for a pack in the morning.

SHOT 3 — hold
Tick as you pack. If something is missing, mark it and write what you have put in instead — do not leave it blank and do not substitute silently. That note reaches the surgeon tonight. A surgeon told at six in the evening will adapt. A surgeon who discovers it at incision will not, and should not have to.

SHOT 4 — push in
When a patient brings a code, type it in and her pack comes up. Record the hand-over so the theatre knows it has left you.

SHOT 5 — pull back
One thing to do tomorrow: clear your list before you go home, not before you go to lunch.
```

```
VIDEO 25 - CONSUMABLE PACKS (2 min 30 s)
SCENES: 1 (0:00-0:18) Phone buzzing in the store at 17:40: "new consumables pre-pack request". 2 (0:18-1:10) Consumable Packs screen with tomorrow's cases and item lists. 3 (1:10-1:55) Items ticked; one unavailable with a substitution note; pack marked ready; readiness board updating. 4 (1:55-2:20) Patient presenting a code; pack found and handed over. 5 (2:20-2:30) Labelled packs on a shelf.
NARRATION: paste the narration column, in order.
```

## V26 · Procurement, vendors and finance

Procurement officer | Finance | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A meeting where somebody is asked how much a class of consumable costs per month and cannot answer. | *The question is reasonable and the answer is somewhere in six spreadsheets. By the time it is assembled, the decision has been taken without it.* |
| 2 · 0:20–1:05 Inventory desk | Inventory Desk: low stock and reorder suggestions driven by actual consumption. | *Inventory Desk shows what is running down and what consumption says you will need, from movements that were recorded as the work happened rather than from an estimate at month end. Order against the trend rather than against the last complaint.* |
| 3 · 1:05–1:50 Vendors | Vendor Accounts: suppliers, orders, deliveries, outstanding balances. | *Vendor Accounts holds suppliers, what was ordered, what arrived and what is owed. Keeping deliveries recorded here is what makes the next negotiation short, because the history is not in dispute.* |
| 4 · 1:50–2:35 Finance and billing | Finance Desk; then Theatre Billing with a completed case and its itemised charges; then Cost Estimates for a planned case. | *Finance Desk and Theatre Billing turn what was actually used into what is actually charged, case by case, instead of a standard figure that is wrong in both directions. Cost Estimates does the same in advance, so a patient can be told what to expect before the day, which is the single most humane thing in this part of the system.* |
| 5 · 2:35–3:00 Close | The same meeting, answered from a phone in ten seconds. Presenter. | *One thing to do this week: open Supply Reports and look at your three fastest-moving items.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A meeting where somebody is asked how much a class of consumable costs per month and cannot answer.
2. **Shot 2** (push in) — Inventory Desk: low stock and reorder suggestions driven by actual consumption.
3. **Shot 3** (push in) — Vendor Accounts: suppliers, orders, deliveries, outstanding balances.
4. **Shot 4** (push in) — Finance Desk; then Theatre Billing with a completed case and its itemised charges; then Cost Estimates for a planned case.
5. **Shot 5** (hold, worth animating) — The same meeting, answered from a phone in ten seconds. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
The question is reasonable and the answer is somewhere in six spreadsheets. By the time it is assembled, the decision has been taken without it.

SHOT 2 — push in
Inventory Desk shows what is running down and what consumption says you will need, from movements that were recorded as the work happened rather than from an estimate at month end. Order against the trend rather than against the last complaint.

SHOT 3 — push in
Vendor Accounts holds suppliers, what was ordered, what arrived and what is owed. Keeping deliveries recorded here is what makes the next negotiation short, because the history is not in dispute.

SHOT 4 — push in
Finance Desk and Theatre Billing turn what was actually used into what is actually charged, case by case, instead of a standard figure that is wrong in both directions. Cost Estimates does the same in advance, so a patient can be told what to expect before the day, which is the single most humane thing in this part of the system.

SHOT 5 — hold
One thing to do this week: open Supply Reports and look at your three fastest-moving items.
```

```
VIDEO 26 - PROCUREMENT, VENDORS AND FINANCE (3 min)
SCENES: 1 (0:00-0:20) A meeting where a monthly cost question cannot be answered. 2 (0:20-1:05) Inventory Desk with consumption-driven reorder suggestions. 3 (1:05-1:50) Vendor Accounts: suppliers, orders, deliveries, balances. 4 (1:50-2:35) Finance Desk; Theatre Billing itemised for a completed case; Cost Estimates for a planned case. 5 (2:35-3:00) The same meeting, answered from a phone.
NARRATION: paste the narration column, in order.
```

## V27 · Porters

Porter | 2 min 30 s

**PRODUCTION NOTE**

Film entirely on a phone, walking, one-handed, in corridors. Nothing in this video should require sitting down or reading closely. Keep sentences very short.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A porter halfway down a corridor, turning back because he has been sent to the wrong ward. | *Wrong ward. Second time today. Both times somebody told him in a corridor, and both times they were remembering, not reading.* |
| 2 · 0:18–1:05<br>Your jobs | Phone alerting; a transfer request showing patient name, ward, bed, destination theatre and how urgent it is. Accepted with one tap. | *Your phone tells you instead. The patient, the ward, the bed, which theatre, and how urgent. Tap to accept it so the theatre knows it is being done and stops asking. That one tap saves the nurses ringing round to find out whether anybody is coming.* |
| 3 · 1:05–1:50<br>On the way | Patient Transfers screen: collected, then delivered, each tapped as it happens with the time recorded. | *Two more taps. Collected when you have the patient. Delivered when you hand over. That is the whole job in the app. It builds the honest record of how long transfers take, which is what gets more porters allocated when the list is heavy — nobody can argue with times that were recorded as they happened.* |
| 4 · 1:50–2:20<br>Announcements | Corridor speaker announcing a patient by name and ward; the porter already walking. | *You will also hear it out loud. The radio announces the patient and the ward, so you often know before you look at the phone.* |
| 5 · 2:20–2:30<br>Close | Delivering a patient calmly; presenter. | *One thing to do tomorrow: accept the job on your phone before you start walking.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — A porter halfway down a corridor, turning back because he has been sent to the wrong ward.
2. **Shot 2** (hold, worth animating) — Phone alerting; a transfer request showing patient name, ward, bed, destination theatre and how urgent it is. Accepted with one tap.
3. **Shot 3** (hold) — Patient Transfers screen: collected, then delivered, each tapped as it happens with the time recorded.
4. **Shot 4** (pan left, worth animating) — Corridor speaker announcing a patient by name and ward; the porter already walking.
5. **Shot 5** (pull back, worth animating) — Delivering a patient calmly; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
Wrong ward. Second time today. Both times somebody told him in a corridor, and both times
they were remembering, not reading.

SHOT 2 — hold
Your phone tells you instead. The patient, the ward, the bed, which theatre, and how urgent.
Tap to accept it so the theatre knows it is being done and stops asking. That one tap saves
the nurses ringing round to find out whether anybody is coming.

SHOT 3 — hold
Two more taps. Collected when you have the patient. Delivered when you hand over. That is the
whole job in the app. It builds the honest record of how long transfers take, which is what
gets more porters allocated when the list is heavy — nobody can argue with times that were
recorded as they happened.

SHOT 4 — pan left
You will also hear it out loud. The radio announces the patient and the ward, so you often
know before you look at the phone.

SHOT 5 — pull back
One thing to do tomorrow: accept the job on your phone before you start walking.
```

```
VIDEO 27 - PORTERS (2 min 30 s)
STYLE: shot entirely on a phone, walking, one-handed, in corridors. Very short sentences.
SCENES: 1 (0:00-0:18) Porter turning back, sent to the wrong ward. 2 (0:18-1:05) Phone alert with
patient, ward, bed, destination theatre, urgency; accepted with one tap. 3 (1:05-1:50) Patient
Transfers: collected, then delivered, times recorded. 4 (1:50-2:20) Corridor speaker announcing
patient and ward; porter already walking. 5 (2:20-2:30) Calm delivery.
NARRATION: paste the narration column, in order.
```

## Part 7 — Engineering and utilities

Power, oxygen, water, equipment, laundry and meals. None of these departments see a patient, and every one of them can stop the list within a minute.

## V28 · Biomedical engineering

Biomedical engineer | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | An engineer stopped in a corridor by three people in turn, each describing a different fault from memory. | *Three faults, three corridors, no theatre numbers and no serial numbers. By the time he reaches the workshop he has one of them clearly and two of them approximately.* |
| 2 · 0:20–1:10 Fault alerts | Fault Alerts list: each fault with theatre, equipment, who reported it, the time and a photograph. One opened, accepted, and marked in progress. | *Fault Alerts is that corridor, written down. Each one carries the theatre, the equipment, who reported it, when, and usually a photograph. Accept it so the reporter can see somebody has it — that single act stops the same fault being reported four more times by four more people.* |
| 3 · 1:10–2:00 Equipment register | Equipment screen: devices with status, location, service history. A device set to out of service; theatre readiness reflecting it immediately. | *The Equipment register holds every device, where it is, whether it works and what has been done to it. When you take something out of service, set it here. The theatre readiness board changes at once, so the coordinator plans around it tonight instead of discovering it at eight tomorrow. Setting it back is just as important — a machine that is fixed but still marked broken is a machine nobody uses.* |
| 4 · 2:00–2:40 Checkout and history | Equipment Checkout showing where a shared device is; then a service history with dates and interventions. | *Checkout tells you which room a shared device is in without opening six doors. And the service history is the argument for replacement: a machine with nine interventions in a year is a case you can make on paper rather than by insisting.* |
| 5 · 2:40–3:00 Close | Fault closed; the reporter's screen showing it resolved. Presenter. | *One thing to do tomorrow: accept every new fault within the hour, even if you cannot start it yet.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan left, worth animating) — An engineer stopped in a corridor by three people in turn, each describing a different fault from memory.
2. **Shot 2** (hold) — Fault Alerts list: each fault with theatre, equipment, who reported it, the time and a photograph. One opened, accepted, and marked in progress.
3. **Shot 3** (hold) — Equipment screen: devices with status, location, service history. A device set to out of service; theatre readiness reflecting it immediately.
4. **Shot 4** (push in) — Equipment Checkout showing where a shared device is; then a service history with dates and interventions.
5. **Shot 5** (hold) — Fault closed; the reporter's screen showing it resolved. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan left
Three faults, three corridors, no theatre numbers and no serial numbers. By the time he
reaches the workshop he has one of them clearly and two of them approximately.

SHOT 2 — hold
Fault Alerts is that corridor, written down. Each one carries the theatre, the equipment, who
reported it, when, and usually a photograph. Accept it so the reporter can see somebody has
it — that single act stops the same fault being reported four more times by four more people.

SHOT 3 — hold
The Equipment register holds every device, where it is, whether it works and what has been
done to it. When you take something out of service, set it here. The theatre readiness board
changes at once, so the coordinator plans around it tonight instead of discovering it at
eight tomorrow. Setting it back is just as important — a machine that is fixed but still
marked broken is a machine nobody uses.

SHOT 4 — push in
Checkout tells you which room a shared device is in without opening six doors. And the
service history is the argument for replacement: a machine with nine interventions in a year
is a case you can make on paper rather than by insisting.

SHOT 5 — hold
One thing to do tomorrow: accept every new fault within the hour, even if you cannot start it
yet.
```

```
VIDEO 28 - BIOMEDICAL ENGINEERING (3 min)
SCENES: 1 (0:00-0:20) Engineer stopped by three people describing faults from memory. 2 (0:20-
1:10) Fault Alerts with theatre, equipment, reporter, time, photograph; one accepted and set in
progress. 3 (1:10-2:00) Equipment register; a device set out of service; readiness board
updating. 4 (2:00-2:40) Equipment Checkout location; service history. 5 (2:40-3:00) Fault closed;
reporter sees it resolved.
NARRATION: paste the narration column, in order.
```

## V29 · Power house

Power plant operator | Works supervisor | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A theatre lit only by an overhead lamp, mid-case, for four seconds. Then full light returns. | *Four seconds. In a laparotomy, four seconds of darkness is a very long time. Whether it happens again this week is decided in a room most theatre staff have never visited.* |
| 2 · 0:20–1:05 Status | Power Status: mains, generators, fuel levels, load, and which theatres are covered. Updated by the operator. | *Power Status is what the theatre suite sees about you. Mains, generators, fuel, load. Update it as things change. When theatre staff can see the fuel level themselves, they stop telephoning to ask, and they plan a long case around it instead of starting and hoping.* |
| 3 · 1:05–1:50 Maintenance and readiness | Power Maintenance: scheduled servicing and test runs logged. Then Power Readiness showing green for the day's list. | *Log test runs and servicing in Power Maintenance as you do them. Power Readiness then answers, for today's list, whether the suite is properly backed. A planned test run recorded here means the theatre knows about it in advance. The same test run unannounced is an incident.* |
| 4 · 1:50–2:20 Raising a problem | Fuel below threshold; an alert raised; the theatre manager notified. | *If fuel is low or a generator is unreliable, raise it here rather than reporting it upward verbally. It reaches the theatre manager with a time against it, and a shortage that is on record at ten in the morning gets solved. The same shortage mentioned in passing does not.* |
| 5 · 2:20–2:30 Close | Steady lights; presenter. | *One thing to do tomorrow: update the fuel level at the start of every shift.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (pan right) — A theatre lit only by an overhead lamp, mid-case, for four seconds. Then full light returns.
2. **Shot 2** (push in) — Power Status: mains, generators, fuel levels, load, and which theatres are covered. Updated by the operator.
3. **Shot 3** (hold) — Power Maintenance: scheduled servicing and test runs logged. Then Power Readiness showing green for the day's list.
4. **Shot 4** (push in) — Fuel below threshold; an alert raised; the theatre manager notified.
5. **Shot 5** (pull back, worth animating) — Steady lights; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — pan right
Four seconds. In a laparotomy, four seconds of darkness is a very long time. Whether it
happens again this week is decided in a room most theatre staff have never visited.

SHOT 2 — push in
Power Status is what the theatre suite sees about you. Mains, generators, fuel, load. Update
it as things change. When theatre staff can see the fuel level themselves, they stop
telephoning to ask, and they plan a long case around it instead of starting and hoping.

SHOT 3 — hold
Log test runs and servicing in Power Maintenance as you do them. Power Readiness then
answers, for today's list, whether the suite is properly backed. A planned test run recorded
here means the theatre knows about it in advance. The same test run unannounced is an
incident.

SHOT 4 — push in
If fuel is low or a generator is unreliable, raise it here rather than reporting it upward
verbally. It reaches the theatre manager with a time against it, and a shortage that is on
record at ten in the morning gets solved. The same shortage mentioned in passing does not.

SHOT 5 — pull back
One thing to do tomorrow: update the fuel level at the start of every shift.
```

```
VIDEO 29 - POWER HOUSE (2 min 30 s)
SCENES: 1 (0:00-0:20) Theatre briefly dark mid-case, then full light. 2 (0:20-1:05) Power Status:
mains, generators, fuel, load, theatres covered. 3 (1:05-1:50) Power Maintenance log; Power
Readiness green for today. 4 (1:50-2:20) Low fuel alert raised; theatre manager notified. 5
(2:20-2:30) Steady lights.
NARRATION: paste the narration column, in order.
```

## V30 · Oxygen control

Oxygen unit supervisor | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A cylinder gauge in the red, discovered by a technician at the start of a list. | *Discovered at ten to eight, by the person who needed it. There were four hours yesterday in which this was a small logistical problem.* |
| 2 · 0:18–1:10<br>Stock and supply | Oxygen Control: cylinders by location and state, manifold pressure, consumption trend. | *Oxygen Control holds what you have, where it is and how fast it is going. Full, in use, empty, awaiting collection. Update as cylinders move. The consumption trend is the part worth watching — it tells you about a shortage days before the gauge does.* |
| 3 · 1:10–1:55<br>Theatre visibility | The theatre readiness board showing an oxygen state; a theatre marked amber because supply is low. | *What you record appears on theatre readiness. That means a low supply is a decision the theatre makes in advance rather than a discovery it makes at the door, and it means nobody has to walk down to ask you.* |
| 4 · 1:55–2:20<br>Escalation | Alert raised for delayed delivery; theatre manager and administration notified. | *If a delivery is late, raise it here. It goes to the theatre manager and to administration together, with a timestamp, so the escalation is already documented if it has to go further.* |
| 5 · 2:20–2:30<br>Close | Full cylinders in place before the list; presenter. | *One thing to do tomorrow: record every cylinder change as you make it.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold, worth animating) — A cylinder gauge in the red, discovered by a technician at the start of a list.
2. **Shot 2** (push in) — Oxygen Control: cylinders by location and state, manifold pressure, consumption trend.
3. **Shot 3** (hold) — The theatre readiness board showing an oxygen state; a theatre marked amber because supply is low.
4. **Shot 4** (push in) — Alert raised for delayed delivery; theatre manager and administration notified.
5. **Shot 5** (hold, worth animating) — Full cylinders in place before the list; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Discovered at ten to eight, by the person who needed it. There were four hours yesterday in
which this was a small logistical problem.

SHOT 2 — push in
Oxygen Control holds what you have, where it is and how fast it is going. Full, in use,
empty, awaiting collection. Update as cylinders move. The consumption trend is the part worth
watching — it tells you about a shortage days before the gauge does.

SHOT 3 — hold
What you record appears on theatre readiness. That means a low supply is a decision the
theatre makes in advance rather than a discovery it makes at the door, and it means nobody
has to walk down to ask you.

SHOT 4 — push in
If a delivery is late, raise it here. It goes to the theatre manager and to administration
together, with a timestamp, so the escalation is already documented if it has to go further.

SHOT 5 — hold
One thing to do tomorrow: record every cylinder change as you make it.
```

```
VIDEO 30 - OXYGEN CONTROL (2 min 30 s)
SCENES: 1 (0:00-0:18) Cylinder gauge in the red found at the start of a list. 2 (0:18-1:10)
Oxygen Control: cylinders by location and state, manifold pressure, consumption trend. 3 (1:10-
1:55) Readiness board showing an amber oxygen state. 4 (1:55-2:20) Late-delivery alert raised. 5
(2:20-2:30) Full cylinders in place.
NARRATION: paste the narration column, in order.
```

## V31 · Plumbing and water

Plumber | Plumbing / water supervisor | 2 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18 Hook | A scrub sink running to a trickle while a surgeon waits to scrub. | *You cannot start a list without water at the scrub sink. This is the least glamorous screen in the whole application and one of the few that can stop everything.* |
| 2 · 0:18–1:05 Status | Plumbing & Water: tank levels, pump status, supply to each theatre area, faults. | *Record tank levels, pump status and supply to each area. When it is on this screen the theatre suite can see it, which is the whole point — a scrub room that will be dry at nine tomorrow is something the coordinator can work around tonight.* |
| 3 · 1:05–1:45 Faults | A leak raised with a photograph and location; assigned; then closed. | *Raise faults with a photograph and the exact location. Close them when they are done, because an open job that is actually finished makes every other number on the screen untrustworthy.* |
| 4 · 1:45–2:00 Close | Water running properly; a surgeon scrubbing. Presenter. | *One thing to do tomorrow: record tank levels at the same time every morning.* |

### PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — A scrub sink running to a trickle while a surgeon waits to scrub.
2. **Shot 2** (push in) — Plumbing & Water: tank levels, pump status, supply to each theatre area, faults.
3. **Shot 3** (push in) — A leak raised with a photograph and location; assigned; then closed.
4. **Shot 4** (pull back, worth animating) — Water running properly; a surgeon scrubbing. Presenter.

### PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 4 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
You cannot start a list without water at the scrub sink. This is the least glamorous screen in the whole application and one of the few that can stop everything.

SHOT 2 — push in
Record tank levels, pump status and supply to each area. When it is on this screen the theatre suite can see it, which is the whole point — a scrub room that will be dry at nine tomorrow is something the coordinator can work around tonight.

SHOT 3 — push in
Raise faults with a photograph and the exact location. Close them when they are done, because an open job that is actually finished makes every other number on the screen untrustworthy.

SHOT 4 — pull back
One thing to do tomorrow: record tank levels at the same time every morning.
```

VIDEO 31 - PLUMBING AND WATER (2 min)
SCENES: 1 (0:00-0:18) Scrub sink running to a trickle while a surgeon waits. 2 (0:18-1:05) Plumbing & Water: tank levels, pumps, supply by area, faults. 3 (1:05-1:45) A leak raised with photograph and location, assigned, then closed. 4 (1:45-2:00) Water running; surgeon scrubbing.
NARRATION: paste the narration column, in order.

## V32 · Laundry, cleaning and meals

Laundry staff | Cleaner | Cafeteria manager | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A theatre between cases. Everything is ready except that the room has not been turned over, and nobody has been told the previous case finished. | *The case finished eleven minutes ago. The next patient is in the holding area. Nobody has told the person whose job it is to turn the room over, so eleven minutes is going to become thirty.* |
| 2 · 0:20–1:05 Cleaning turnaround | A cleaner's phone alerting that a theatre needs turning over; accepted; marked complete; the readiness board turning green. | *When a case finishes, the room shows as needing cleaning, and that reaches you rather than waiting to be noticed. Accept it, do it, mark it done. The readiness board turns green and the coordinator can send for the next patient. Those two taps are worth twenty minutes a day on a busy list.* |
| 3 · 1:05–1:50 Laundry | Laundry screen: linen sent, received, in process; stock of gowns and drapes; a shortage flagged. | *Laundry records what has gone out, what has come back and what is in stock. Flag a shortage as soon as you see it. Gowns and drapes are the kind of thing that is fine, fine, fine, and then stops a list on a Thursday morning — and the warning is always available a day before, on this screen.* |
| 4 · 1:50–2:40 Meals | Theatre Meals showing numbers expected by shift; then Order Meal used by a nurse on a phone. | *Theatre Meals tells you how many people are actually on duty, from the roster, rather than from a guess. Staff order from their phones during the list. It sounds small next to the rest of this system. It is not: a team that has eaten runs a safer afternoon list, and a long emergency case is where that shows.* |
| 5 · 2:40–3:00 Close | Fast, calm turnover between cases. Presenter. | *One thing to do tomorrow: mark the room done the moment it is done, not when you next pass a screen.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A theatre between cases. Everything is ready except that the room has not been turned over, and nobody has been told the previous case finished.
2. **Shot 2** (hold) — A cleaner's phone alerting that a theatre needs turning over; accepted; marked complete; the readiness board turning green.
3. **Shot 3** (hold) — Laundry screen: linen sent, received, in process; stock of gowns and drapes; a shortage flagged.
4. **Shot 4** (hold, worth animating) — Theatre Meals showing numbers expected by shift; then Order Meal used by a nurse on a phone.
5. **Shot 5** (pull back, worth animating) — Fast, calm turnover between cases. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
The case finished eleven minutes ago. The next patient is in the holding area. Nobody has
told the person whose job it is to turn the room over, so eleven minutes is going to become
thirty.

SHOT 2 — hold
When a case finishes, the room shows as needing cleaning, and that reaches you rather than
waiting to be noticed. Accept it, do it, mark it done. The readiness board turns green and
the coordinator can send for the next patient. Those two taps are worth twenty minutes a day
on a busy list.

SHOT 3 — hold
Laundry records what has gone out, what has come back and what is in stock. Flag a shortage
as soon as you see it. Gowns and drapes are the kind of thing that is fine, fine, fine, and
then stops a list on a Thursday morning — and the warning is always available a day before,
on this screen.

SHOT 4 — hold
Theatre Meals tells you how many people are actually on duty, from the roster, rather than
from a guess. Staff order from their phones during the list. It sounds small next to the rest
of this system. It is not: a team that has eaten runs a safer afternoon list, and a long
emergency case is where that shows.

SHOT 5 — pull back
One thing to do tomorrow: mark the room done the moment it is done, not when you next pass a
screen.
```

```
VIDEO 32 - LAUNDRY, CLEANING AND MEALS (3 min)
SCENES: 1 (0:00-0:20) Theatre between cases, not turned over, next patient waiting. 2 (0:20-1:05)
Cleaner's phone alert; accepted; marked complete; readiness board turning green. 3 (1:05-1:50)
Laundry: linen sent, received, in process; stock; shortage flagged. 4 (1:50-2:40) Theatre Meals
numbers from the roster; a nurse ordering from a phone. 5 (2:40-3:00) Fast calm turnover.
NARRATION: paste the narration column, in order.
```

## Part 8 — Leadership and administration

Theatre managers, chairmen, medical directors and system administrators. These videos are about seeing the whole suite, and about the responsibilities that come with being able to.

## V33 · The theatre manager's day

Theatre manager | Theatre chairman | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A manager's office at 07:30. Three people waiting at the door, each with a different problem. | *Half past seven, and three people are already at the door. By nine she will have heard about the fourth problem, which started at six and which she could have seen last night.* |
| 2 · 0:20–1:10<br>The evening view | Theatre Readiness and Case Pack Readiness open side by side the previous evening. Two amber rows. | *The job starts the night before, with two screens. Theatre Readiness for the rooms, Case Pack Readiness for the cases. Two amber rows at six in the evening are two phone calls. The same two rows at eight in the morning are a delayed list and three people at your door.* |
| 3 · 1:10–2:00<br>Allocation | Theatre Allocation and Surgical Unit Calendar: units against theatres across the week; a unit moved to a different room. | *Allocation puts units into rooms across the week, and the Surgical Unit Calendar is how the units see it. Change it here and everyone sees the change — there is no separate list to circulate and no version confusion, which is where most of the arguments used to come from.* |
| 4 · 2:00–2:50<br>Live day | Theatre Operations: live view of all theatres, cases in progress, milestones, delays highlighted. Emergency Response showing an unassigned emergency. | *Through the day, Theatre Operations is the suite in real time: what is running, what has started late and by how much. Emergency Response shows any emergency without a theatre or a team — that screen is the one to keep open, because an emergency nobody has taken is the most expensive thing on your list.* |
| 5 · 2:50–3:20<br>Staff | Staff Availability and Duty Flyers; a gap in tomorrow's cover highlighted. | *Staff Availability shows who is available and where the gaps are. Duty Flyers publishes the day's assignments so nobody arrives asking where they are working.* |
| 6 · 3:20–3:30<br>Close | Quiet office at 07:30, nobody waiting. Presenter. | *One thing to do tomorrow: look at both readiness screens at six in the evening, every evening.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A manager's office at 07:30. Three people waiting at the door, each with a different problem.
2. **Shot 2** (push in) — Theatre Readiness and Case Pack Readiness open side by side the previous evening. Two amber rows.
3. **Shot 3** (push in) — Theatre Allocation and Surgical Unit Calendar: units against theatres across the week; a unit moved to a different room.
4. **Shot 4** (push in) — Theatre Operations: live view of all theatres, cases in progress, milestones, delays highlighted. Emergency Response showing an unassigned emergency.
5. **Shot 5** (push in, worth animating) — Staff Availability and Duty Flyers; a gap in tomorrow's cover highlighted.
6. **Shot 6** (pull back, worth animating) — Quiet office at 07:30, nobody waiting. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
Half past seven, and three people are already at the door. By nine she will have heard about the fourth problem, which started at six and which she could have seen last night.

SHOT 2 — push in
The job starts the night before, with two screens. Theatre Readiness for the rooms, Case Pack Readiness for the cases. Two amber rows at six in the evening are two phone calls. The same two rows at eight in the morning are a delayed list and three people at your door.

SHOT 3 — push in
Allocation puts units into rooms across the week, and the Surgical Unit Calendar is how the units see it. Change it here and everyone sees the change — there is no separate list to circulate and no version confusion, which is where most of the arguments used to come from.

SHOT 4 — push in
Through the day, Theatre Operations is the suite in real time: what is running, what has started late and by how much. Emergency Response shows any emergency without a theatre or a team — that screen is the one to keep open, because an emergency nobody has taken is the most expensive thing on your list.

SHOT 5 — push in
Staff Availability shows who is available and where the gaps are. Duty Flyers publishes the day's assignments so nobody arrives asking where they are working.

SHOT 6 — pull back
One thing to do tomorrow: look at both readiness screens at six in the evening, every evening.
```

```
VIDEO 33 - THE THEATRE MANAGER'S DAY (3 min 30 s)
SCENES: 1 (0:00-0:20) Manager's office 07:30, three people waiting. 2 (0:20-1:10) Theatre Readiness and Case Pack Readiness side by side the previous evening, two amber rows. 3 (1:10-2:00) Theatre Allocation and Surgical Unit Calendar; a unit moved. 4 (2:00-2:50) Theatre Operations live view with delays; Emergency Response showing an unassigned emergency. 5 (2:50-3:20) Staff Availability and Duty Flyers with a cover gap. 6 (3:20-3:30) Quiet office, nobody waiting.
NARRATION: paste the narration column, in order.
```

## V34 · Publishing a departmental roster

Rota holders | Department heads | 3 min 30 s

**SAY THIS PLAINLY IN THE VIDEO**

A roster that is entered but **not published** does nothing at all. Draft rows are invisible to booking, to coverage and to the boards. Publishing is the step that makes it real, and forgetting it is the single most consequential mistake in this module.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:25 Hook | A completed roster on screen with twenty-two rows, all marked draft. Beside it, tomorrow's theatre list showing "not yet assigned" against every case. | *Twenty-two people, entered correctly, for tomorrow. And tomorrow's list says nobody is assigned. The work was done. The last button was not pressed, and until it is, none of this exists.* |
| 2 · 0:25–1:15 Entering | Duty Roster on a phone. Department chosen. A staff member added by typing three letters into a search-to-select field. Shift chosen: Electives or Call/Emergencies. Assignment chosen — a surgical specialty for anaesthetists, a theatre for technicians. | *Open Duty Roster and pick your department. Add a person by typing part of their name — the list finds them. Choose the shift: Electives, or Call and Emergencies. Then the assignment, and this is the field that matters most. For anaesthetists it is the surgical specialty they are covering. For technicians it is the theatre. That single field is what lets a booked case find the right person, so an empty one is a case that will read as unassigned tomorrow.* |
| 3 · 1:15–2:00 Mobile first | The same form on a small phone screen, all fields visible, nothing cut off. Rows being added quickly, one after another. | *Do it from your phone if that is easier. Every field fits on the smallest screen — nothing is hidden behind a scroll, because most rosters in this hospital are submitted from a phone, often standing up, often at night.* |
| 4 · 2:00–2:50 Publish | The pending-changes count showing 22 drafts and 1 staged removal. Publish pressed. Version number increments; rows turn published; the coverage screen fills with names. | *Now the important part. Your changes sit as drafts, and the counter tells you how many are waiting. Press Publish. The version number goes up, a permanent snapshot is taken so you can see exactly what was published and by whom, and only now does the rest of the hospital see it. Watch the coverage screen fill up. And notice: cases already booked pick up their proper anaesthetist automatically. You do not go back through them.* |
| 5 · 2:50–3:20 Changes and history | A staged removal; a new version published; the versions list showing previous publications. | *Later changes work the same way: amend, then publish again. Every version is kept, so if somebody asks what the roster said on a particular day, the answer is not a memory.* |
| 6 · 3:20–3:30 Close | All-green coverage; presenter. | *One thing to do tomorrow: after you enter a roster, look for the pending count. If it is not zero, you have not finished.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A completed roster on screen with twenty-two rows, all marked draft. Beside it, tomorrow's theatre list showing "not yet assigned" against every case.
2. **Shot 2** (hold, worth animating) — Duty Roster on a phone. Department chosen. A staff member added by typing three letters into a search-to-select field. Shift chosen: Electives or Call/Emergencies. Assignment chosen — a surgical specialty for anaesthetists, a theatre for technicians.
3. **Shot 3** (hold) — The same form on a small phone screen, all fields visible, nothing cut off. Rows being added quickly, one after another.
4. **Shot 4** (hold) — The pending-changes count showing 22 drafts and 1 staged removal. Publish pressed. Version number increments; rows turn published; the coverage screen fills with names.
5. **Shot 5** (hold) — A staged removal; a new version published; the versions list showing previous publications.
6. **Shot 6** (pull back, worth animating) — All-green coverage; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Twenty-two people, entered correctly, for tomorrow. And tomorrow's list says nobody is
assigned. The work was done. The last button was not pressed, and until it is, none of this
exists.

SHOT 2 — hold
Open Duty Roster and pick your department. Add a person by typing part of their name — the
list finds them. Choose the shift: Electives, or Call and Emergencies. Then the assignment,
and this is the field that matters most. For anaesthetists it is the surgical specialty they
are covering. For technicians it is the theatre. That single field is what lets a booked case
find the right person, so an empty one is a case that will read as unassigned tomorrow.

SHOT 3 — hold
Do it from your phone if that is easier. Every field fits on the smallest screen — nothing is
hidden behind a scroll, because most rosters in this hospital are submitted from a phone,
often standing up, often at night.

SHOT 4 — hold
Now the important part. Your changes sit as drafts, and the counter tells you how many are
waiting. Press Publish. The version number goes up, a permanent snapshot is taken so you can
see exactly what was published and by whom, and only now does the rest of the hospital see
it. Watch the coverage screen fill up. And notice: cases already booked pick up their proper
anaesthetist automatically. You do not go back through them.

SHOT 5 — hold
Later changes work the same way: amend, then publish again. Every version is kept, so if
somebody asks what the roster said on a particular day, the answer is not a memory.

SHOT 6 — pull back
One thing to do tomorrow: after you enter a roster, look for the pending count. If it is not
zero, you have not finished.
```

```
VIDEO 34 - PUBLISHING A DEPARTMENTAL ROSTER (3 min 30 s)
SCENES: 1 (0:00-0:25) Twenty-two draft rows beside a theatre list reading "not yet assigned" on
```

```
every case. 2 (0:25-1:15) Duty Roster on a phone: department, staff added by type-to-search,
shift Electives or Call/Emergencies, assignment field (specialty for anaesthetists, theatre for
technicians). 3 (1:15-2:00) Same form on a very small phone screen, all fields visible; rows
added quickly. 4 (2:00-2:50) Pending count 22 drafts and 1 staged removal; Publish pressed;
version increments; coverage screen fills with names. 5 (2:50-3:20) A staged removal; a new
version; version history list. 6 (3:20-3:30) All-green coverage.
NARRATION: paste the narration column, in order.
```

## V35 · Reports and analytics

Chairman | CMAC | Medical director | 3 min

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20 Hook | A management meeting. Two people disagreeing about how many cases were cancelled last month; both are confident. | *Both of them are certain, and they differ by nineteen cases. Neither is lying. They are both remembering, and memory is not a data source.* |
| 2 · 0:20–1:10 Reports | Reports & Analytics: case volume by unit, on-time starts, turnover times, cancellations by reason, theatre utilisation. | *Reports is built from timestamps recorded as the work happened. Case volume by unit. How many lists started on time. How long turnovers actually take. Cancellations, grouped by reason — and that last one is the most useful screen in the building, because it turns a complaint into a list you can act on.* |
| 3 · 1:10–2:00 Performance and people | Theatre Ops Performance; then Staff Effectiveness. | *Performance shows where the time goes across the suite: which stage of the day loses it and where the queue forms. Staff Effectiveness does the same for people, and it deserves one caution said out loud. Use it to find where support is needed, not to rank colleagues. The moment staff believe it is a league table, they will start recording things late, and then every number here becomes worthless — including the ones you actually need.* |
| 4 · 2:00–2:40 Research and presenting | Research & Analytics with a filtered cohort exported; then Presentation mode showing full-screen charts. | *Research lets you build a cohort and export it for study. Presentation puts the same figures on a screen for a meeting, so you are arguing from the system rather than from a slide somebody typed last night.* |
| 5 · 2:40–3:00 Close | The meeting from scene 1, now looking at one screen together. Presenter. | *One thing to do this month: open cancellations by reason and fix the top one. It is almost always the cheapest thing on the list.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A management meeting. Two people disagreeing about how many cases were cancelled last month; both are confident.
2. **Shot 2** (push in) — Reports & Analytics: case volume by unit, on-time starts, turnover times, cancellations by reason, theatre utilisation.
3. **Shot 3** (hold) — Theatre Ops Performance; then Staff Effectiveness.
4. **Shot 4** (hold) — Research & Analytics with a filtered cohort exported; then Presentation mode showing full-screen charts.
5. **Shot 5** (hold) — The meeting from scene 1, now looking at one screen together. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
Both of them are certain, and they differ by nineteen cases. Neither is lying. They are both
remembering, and memory is not a data source.

SHOT 2 — push in
Reports is built from timestamps recorded as the work happened. Case volume by unit. How many
lists started on time. How long turnovers actually take. Cancellations, grouped by reason —
and that last one is the most useful screen in the building, because it turns a complaint
into a list you can act on.

SHOT 3 — hold
Performance shows where the time goes across the suite: which stage of the day loses it and
where the queue forms. Staff Effectiveness does the same for people, and it deserves one
caution said out loud. Use it to find where support is needed, not to rank colleagues. The
moment staff believe it is a league table, they will start recording things late, and then
every number here becomes worthless — including the ones you actually need.

SHOT 4 — hold
Research lets you build a cohort and export it for study. Presentation puts the same figures
on a screen for a meeting, so you are arguing from the system rather than from a slide
somebody typed last night.

SHOT 5 — hold
One thing to do this month: open cancellations by reason and fix the top one. It is almost
always the cheapest thing on the list.
```

```
VIDEO 35 - REPORTS AND ANALYTICS (3 min)
SCENES: 1 (0:00-0:20) Management meeting, two people disagreeing on cancellation numbers. 2
(0:20-1:10) Reports & Analytics: volume by unit, on-time starts, turnover, cancellations by
reason, utilisation. 3 (1:10-2:00) Theatre Ops Performance; Staff Effectiveness with the caution
about ranking. 4 (2:00-2:40) Research & Analytics cohort export; Presentation mode. 5 (2:40-3:00)
The meeting now looking at one screen together.
NARRATION: paste the narration column, in order.
```

## V36 · Administering the system

Administrator | System administrator | 3 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:20<br>Hook | A new registrar on her first day, unable to see the booking screen, standing behind somebody else who is booking for her. | *Day one. She cannot see the booking screen, so somebody else is booking under their own name for her. Every case she does this week will have the wrong person's name on it, and that is an administration problem, not a clinical one.* |
| 2 · 0:20–1:10<br>Users | User Management: pending approvals, a new user approved, a role assigned, phone number and department checked. | *User Management is where staff are approved and given a role. Approve promptly — a clinician who cannot get in will borrow somebody else's login, and from that moment your audit trail is fiction. Check the phone number while you are there. It is what Chase on WhatsApp uses, and a wrong number there wastes somebody's morning.* |
| 3 · 1:10–1:55<br>Module access | Module Access: granting one extra screen to a named user with a reason recorded. | *Roles cover most people, but not everyone. Module Access grants one extra screen to one named person, with a reason. Use it narrowly. The temptation is to give somebody the administrator role because it is faster, and that hands them everything, including things they should never see.* |
| 4 · 1:55–2:40<br>Announcements and radio | Announcements: composing one, choosing when it plays, scheduling it daily. Then the radio playing it at the scheduled time. | *Announcements are what the suite hears. Write it, choose when it plays, and it is spoken at that time — once, at the right time, on the right day. Keep them few. An announcement that plays too often is an announcement people learn to ignore, and one day it will be the emergency alert they ignore.* |
| 5 · 2:40–3:15<br>Theatre music | Theatre Music: uploading files into a category; the library filling; the player in the corner of the screen switching to it. | *Theatre Music is yours too. Upload tracks into categories and they appear in the player in the corner of every screen. It ducks automatically whenever the radio speaks, so it never covers an announcement. One responsibility comes with it: music played in a theatre is a public performance, so use recordings the hospital is entitled to play. The app cannot check that for you.* |
| 6 · 3:15–3:30<br>Close | The registrar from scene 1, booking under her own name. Presenter. | *One thing to do tomorrow: clear the pending user approvals before you do anything else.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (hold) — A new registrar on her first day, unable to see the booking screen, standing behind somebody else who is booking for her.
2. **Shot 2** (hold) — User Management: pending approvals, a new user approved, a role assigned, phone number and department checked.
3. **Shot 3** (hold) — Module Access: granting one extra screen to a named user with a reason recorded.
4. **Shot 4** (push in) — Announcements: composing one, choosing when it plays, scheduling it daily. Then the radio playing it at the scheduled time.
5. **Shot 5** (hold) — Theatre Music: uploading files into a category; the library filling; the player in the corner of the screen switching to it.
6. **Shot 6** (pull back, worth animating) — The registrar from scene 1, booking under her own name. Presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 6 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — hold
Day one. She cannot see the booking screen, so somebody else is booking under their own name for her. Every case she does this week will have the wrong person's name on it, and that is an administration problem, not a clinical one.

SHOT 2 — hold
User Management is where staff are approved and given a role. Approve promptly — a clinician who cannot get in will borrow somebody else's login, and from that moment your audit trail is fiction. Check the phone number while you are there. It is what Chase on WhatsApp uses, and a wrong number there wastes somebody's morning.

SHOT 3 — hold
Roles cover most people, but not everyone. Module Access grants one extra screen to one named person, with a reason. Use it narrowly. The temptation is to give somebody the administrator role because it is faster, and that hands them everything, including things they should never see.

SHOT 4 — push in
Announcements are what the suite hears. Write it, choose when it plays, and it is spoken at that time — once, at the right time, on the right day. Keep them few. An announcement that plays too often is an announcement people learn to ignore, and one day it will be the emergency alert they ignore.

SHOT 5 — hold
Theatre Music is yours too. Upload tracks into categories and they appear in the player in the corner of every screen. It ducks automatically whenever the radio speaks, so it never covers an announcement. One responsibility comes with it: music played in a theatre is a public performance, so use recordings the hospital is entitled to play. The app cannot check that for you.

SHOT 6 — pull back
One thing to do tomorrow: clear the pending user approvals before you do anything else.
```

```
VIDEO 36 - ADMINISTERING THE SYSTEM (3 min 30 s)
SCENES: 1 (0:00-0:20) New registrar unable to see the booking screen, watching someone book for
```

her. 2 (0:20-1:10) User Management: pending approvals, approve, assign role, check phone number. 3 (1:10-1:55) Module Access granting one screen to one named user with a reason. 4 (1:55-2:40) Announcements composed and scheduled; radio playing it at the right time. 5 (2:40-3:15) Theatre Music upload; library filling; player switching source; ducking under an announcement. 6 (3:15-3:30) Registrar booking under her own name.
NARRATION: paste the narration column, in order.

## V37 · Imprest and billing

Imprest holders | Administration | 2 min 30 s

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:18<br>Hook | A drawer of receipts being emptied onto a desk at the end of a quarter. | *Three months of receipts, one afternoon, and at least four of them will be missing. Retirement does not have to be an archaeology exercise.* |
| 2 · 0:18–1:05<br>The register | Imprest Register: an expense entered with amount, purpose and a photographed receipt attached. | *Enter each expense when you make it, and photograph the receipt straight onto the entry. Thirty seconds at the time replaces an afternoon at the end, and the receipt cannot be lost because it is no longer paper.* |
| 3 · 1:05–1:50<br>Position and retirement | Quarterly Position showing spend against allocation; then Retirements assembling a complete return. | *Quarterly Position shows what has been spent against what was allocated, at any moment rather than at the end. Retirements assembles the return from entries that already exist, with the receipts attached.* |
| 4 · 1:50–2:20<br>Duties and reports | Imprest Duties assigning responsibility; Imprest Reports showing a trend across quarters. | *Duties records who is responsible for what, so a query goes to a person rather than to a department. Reports show the trend across quarters, which is the case you make when the allocation is set.* |
| 5 · 2:20–2:30<br>Close | Empty drawer; presenter. | *One thing to do tomorrow: photograph the receipt before you leave the counter.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in) — A drawer of receipts being emptied onto a desk at the end of a quarter.
2. **Shot 2** (push in) — Imprest Register: an expense entered with amount, purpose and a photographed receipt attached.
3. **Shot 3** (push in) — Quarterly Position showing spend against allocation; then Retirements assembling a complete return.
4. **Shot 4** (push in) — Imprest Duties assigning responsibility; Imprest Reports showing a trend across quarters.
5. **Shot 5** (pull back, worth animating) — Empty drawer; presenter.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
Three months of receipts, one afternoon, and at least four of them will be missing.
Retirement does not have to be an archaeology exercise.

SHOT 2 — push in
Enter each expense when you make it, and photograph the receipt straight onto the entry.
Thirty seconds at the time replaces an afternoon at the end, and the receipt cannot be lost
because it is no longer paper.

SHOT 3 — push in
Quarterly Position shows what has been spent against what was allocated, at any moment rather
than at the end. Retirements assembles the return from entries that already exist, with the
receipts attached.

SHOT 4 — push in
Duties records who is responsible for what, so a query goes to a person rather than to a
department. Reports show the trend across quarters, which is the case you make when the
allocation is set.

SHOT 5 — pull back
One thing to do tomorrow: photograph the receipt before you leave the counter.
```

```
VIDEO 37 - IMPREST AND BILLING (2 min 30 s)
SCENES: 1 (0:00-0:18) A drawer of receipts emptied onto a desk at quarter end. 2 (0:18-1:05)
Imprest Register entry with amount, purpose and photographed receipt. 3 (1:05-1:50) Quarterly
Position; Retirements assembled from existing entries. 4 (1:50-2:20) Imprest Duties; Imprest
Reports trend. 5 (2:20-2:30) Empty drawer.
NARRATION: paste the narration column, in order.
```

## V38 · The safety net

All staff | 3 min

### THE BRIEF

The last video in the series, and the one that decides whether staff trust the system with bad news. Tone must be calm and protective throughout.

| SCENE | ON SCREEN | NARRATION |
|---|---|---|
| 1 · 0:00–0:25 Hook | A junior member of staff noticing something wrong, looking at a senior colleague, and saying nothing. | *She saw it. She is the most junior person in the room, and she has decided it is not worth what it would cost to say. Every serious incident in every hospital in the world has this moment in it somewhere.* |
| 2 · 0:25–1:10 Alerts and faults | Alerts screen; a fault raised from a theatre; an emergency alert going out. | *Most things are ordinary and go through Alerts and Fault Alerts. Something is broken, something is missing, something is not safe to use. Raise it with a photograph, and it goes to the people who can act with the time and the place attached. Nothing about that is a complaint. It is how the building learns.* |
| 3 · 1:10–2:00 Anonymous tips | Anonymous Tips: a report being written with no name attached; the submission confirming that no identity was recorded. | *And some things are not ordinary. If you have seen something you are not comfortable putting your name to — unsafe practice, pressure to cut a corner, something worse — Anonymous Tips exists for exactly that. No name is recorded. It reaches the people who must know. Use it when you need it, and use it honestly. It is read.* |
| 4 · 2:00–2:35 Feedback and mortality | Feedback form being submitted; then Mortality Registry and Security Reports. | *Feedback is for the app itself and for how the theatre runs — the suggestions screen is read by the people who build this. The Mortality Registry and Security Reports are the formal records, kept properly so that a review argues about the case and not about what happened.* |
| 5 · 2:35–3:00 Close | The junior member of staff from scene 1, speaking up. The room listening. Presenter, final title card. | *A system is only as safe as the worst thing people are willing to report in it. One thing to do tomorrow: if something today felt wrong and you said nothing, write it down before you go home. That is the whole point of all thirty-eight of these videos.* |

## PRODUCE — STILLS TO PREPARE

One still per shot, in this order. Load them into *Images to animate*. Shots marked *worth animating* are the only ones where AI generation earns its cost — a screen never does.

1. **Shot 1** (push in, worth animating) — A junior member of staff noticing something wrong, looking at a senior colleague, and saying nothing.
2. **Shot 2** (hold) — Alerts screen; a fault raised from a theatre; an emergency alert going out.
3. **Shot 3** (push in) — Anonymous Tips: a report being written with no name attached; the submission confirming that no identity was recorded.
4. **Shot 4** (hold) — Feedback form being submitted; then Mortality Registry and Security Reports.
5. **Shot 5** (hold, worth animating) — The junior member of staff from scene 1, speaking up. The room listening. Presenter, final title card.

## PRODUCE — PASTE INTO THE NARRATION BOX

Paste exactly as below. The app reads `SHOT n — motion` as a heading and every other line as narration, so this fills the motion and narration fields for all 5 shots in one go. Do not paste the still descriptions — they would be spoken aloud.

```
SHOT 1 — push in
She saw it. She is the most junior person in the room, and she has decided it is not worth what it would cost to say. Every serious incident in every hospital in the world has this moment in it somewhere.

SHOT 2 — hold
Most things are ordinary and go through Alerts and Fault Alerts. Something is broken, something is missing, something is not safe to use. Raise it with a photograph, and it goes to the people who can act with the time and the place attached. Nothing about that is a complaint. It is how the building learns.

SHOT 3 — push in
And some things are not ordinary. If you have seen something you are not comfortable putting your name to — unsafe practice, pressure to cut a corner, something worse — Anonymous Tips exists for exactly that. No name is recorded. It reaches the people who must know. Use it when you need it, and use it honestly. It is read.

SHOT 4 — hold
Feedback is for the app itself and for how the theatre runs — the suggestions screen is read by the people who build this. The Mortality Registry and Security Reports are the formal records, kept properly so that a review argues about the case and not about what happened.

SHOT 5 — hold
A system is only as safe as the worst thing people are willing to report in it. One thing to do tomorrow: if something today felt wrong and you said nothing, write it down before you go home. That is the whole point of all thirty-eight of these videos.
```

```
VIDEO 38 - THE SAFETY NET (3 min)
STYLE: calm and protective. Never accusatory.
SCENES: 1 (0:00-0:25) A junior staff member noticing something wrong and saying nothing. 2 (0:25-1:10) Alerts and Fault Alerts; a fault raised with a photograph; an emergency alert going out. 3 (1:10-2:00) Anonymous Tips written with no name; confirmation that no identity was recorded. 4 (2:00-2:35) Feedback submitted; Mortality Registry; Security Reports. 5 (2:35-3:00) The junior staff member speaking up; the room listening; final title card.
NARRATION: paste the narration column, in order.
```

## Appendix — Production checklist

| BEFORE YOU RENDER | CHECK |
|---|---|
| Consistency | Same presenter, same voice, same lower-third style in all thirty-eight. |
| Device | Phone for anything done standing up; laptop for desk work. Never a desktop screenshot for a phone task. |
| Patient data | Only the fictional names in these scripts. No real folder numbers on screen, ever. |
| Length | Nothing over four minutes. If it runs long, split it. |
| Opening | A real problem in the first eight seconds. No "in this video we will". |
| Ending | Exactly one action for the next working day. |
| Tone | Read V3 scene 4 and V35 scene 3 aloud. If either sounds like a warning, rewrite it. |
| Accuracy | Re-shoot any screen that has changed since filming. A training video showing a screen that no longer exists costs more trust than it builds. |